KENYA MEDICAL TRAINING COLLEGE

FACULTY OF CLINICAL SCIENCES

DEPARTMENT OF CLINICAL MEDICINE

2015/2016 ACADEMIC YEAR

Final Qualifying Examinations for the Award of Diploma in Clinical Medicine and Surgery

JULY 2016

STREAM: Diploma

MODULE: INTERNAL MEDICINE PAPER I

TIME ALLOWED: 3 Hours

SESSION: Morning (09.00 – 12.00 hours)

DATE: MONDAY, 4TH JULY, 2016

GENERAL INSTRUCTIONS

1) Write you examination number in the spaces provided on the answer booklet
2) Read instructions on answer booklet KMTC/QP-08/EAB and question paper carefully
3) Paper I accounts for 40% of the examination scores

SECTION A – SHORT ESSAY/STRUCTURED QUESTIONS – 60 marks

- This section contains **SIX questions**
- **ATTEMPT** all questions

SECTION B – LONG ESSAY QUESTIONS – 40 MARKS

- This section contains THREE questions
- Attempt any TWO questions

MEDICINE PAPER I

FQE 2016

Section A: SHORT ESSAY QUESTIONS – ANSWER all questions

1. Concerning malaria
   a) Enumerate 5 clinical features of complicated malaria (5marks)
   b) Outline the management plan of complicated malaria. (5 marks)

2. A 25 year old male is brought to hospital in coma, with a 2 hours history of ingestion unknown chemical following a domestic quarrel. Upon examination the patient is in coma with constricted pupil and saliva drooling from the mouth.
   a) What is the most likely diagnosis?. (2marks)
   b) Outline the immediate supportive approach to this patient. (4marks)
   c) Outline the definitive treatment to this patient. 4 marks

3. A female patient reports to hospital with a 2 day history of painful swollen left leg. Patient says she travelled 4 days ago from Kisumu to Mombasa by bus. Upon examination left leg is swollen, warm and tender allowed the calf muscle.
   a) State the diagnosis (1 mark)
   b) Give any other 4 predisposing factor to this condition. (2marks)
   c) State the diagnostic test to this condition. (1 mark)
   d) Provide supportive management for this patient (3 marks)
   e) Give the specific treatment for the patient. (4 marks)

4. About cholera
   a) Outline any 4 clinical presentations. (4 marks)
   b) Write the relevant investigation (1mark)
   c) Write its treatment (5 marks)

5. Kasongo 28 years old is brought to you in coma. Relatives report to you that he has been taking a lot of water, eating a lot and passing urine often. He has lost weight. On examination Kasongo is unrousable and has deep regular respirations associated with a sweet smell. Blood pressure is 100/60mmHg and he is severely dehydrated. When you do urgent urinalysis, it shows Ketones 3++

   a) State the most likely diagnosis (1mark)
   b) Outline the fluid management of this patient (5 marks)
   c) Enumerate 4 possible long term complication associated with this condition. (4 marks)

6. Concerning Acute Urinary Tract Infection (UTI)
   a) State 3 predisposing factors. (3marks)
   b) Give any 4 clinical presentations (4 marks)
   c) Write 2 possible urine findings (2 marks)
   d) State any 2 drugs used in its treatment (1 mark)

SECTION B: LONG ESSAY QUESTIONS - ANSWER 2 QUESTIONS ONLY

1. Mr. Kamash who is 45 years old presented to you in Out Patient Department with productive cough lasting for 3 weeks , chest pains and night sweats. He also give a history of weight loss. One examination is wasted.
   a) State your diagnosis. (2marks)
   b) List 4 differential diagnosis. (4 marks)
   c) Outline any 4 relevant investigations. (4 marks)
   d) Treat the above patient. (5 marks)
   e) List 5 possible complication. (5 marks)

2. A 25 year old patient presented with a 2 day history of severe headache, blurred vision, photophobia and body hotness. Upon examination, the patient was sick looking with an auxiliary temperature of 38.5° C . Kernigs sign was positive.
   a) State the most likely diagnosis. (2marks)
   b) List any 4 differential diagnosis. (4marks)
   c) Outline any 4 relevant investigations. (4 marks)
   d) Treat this patient. (6 marks)
   e) List any four (4) complications (4marks)

3. A patient presents with 3 weeks history of headache and dizziness, two episodes of fainting and awareness of heart beat. On examination, the patient is severely pale with a Hb of 4g/dl.
   a) List 6 possible causes of the above condition (6 marks)
   b) Outline any 4 relevant investigations. (4 marks)
   c) Manage the patient. (10 marks)

FILE

KMTC/QP-08/TIS

KENYA MEDICAL TRAINING COLLEGE

PORT REITZ CAMPUS

DEPARTMENT OF CLINICAL MEDICINE

END OF SEMESTER SIX IN 3RD YEAR

FOR THE 2013 CLASS

PAPER:	MEDICINE
DATE:	12TH MAY 2016
TIME:	9.00 A.M. – 12 NOON
DAY:	THURSDAY
PAR A (TRUE/FALSE)	50 MARKS
PART B (MCQ):	20 MARKS
PART C (ESSAY):	30 MARKS
TOTAL MARKS:	100 MARKS
TOTAL NO. OF PAGES:	6

INSTRUCTIONS

1. USE REGISTRATION NUMBERS AND NOT NAMES IN THIS EXAMINATION.
2. ANSWER PART A (TRUE/FALSE) ON THE ANSWER SHEET PROVIDED.
3. ANSWER PART B (MCQ) ON THE ANSWER SHEET PROVIDED.
4. ANSWER PART C ON THE FULLSCAPS PROVIDED.
5. MOBILE PHONES, TEXT BOOKS OR ANY LITERATURE ARE FORBIDDEN IN THE EXAMINATION ROOM.

PART A: MULTIPLE TRUE FALSE QUESTIONS

1) The following are indications for anti venom administration in snake bite:
   a] Cardiogenic shock
   b] Haematuria
   c] Rapid progressive extensive local swelling
   d] Neurotoxicity
   e] Spontaneous systemic bleeding

2) Shahbibi developed symptoms of nausea and profuse vomiting 3 hrs after ingesting some food in a party organized by students in KMTC. Diarrhea was not marked, though she has a lot of abdominal crumps and high fever:
   a] This is definitely amoebiasis
   b] Antiemetic is very necessary
   c] Suspected food should be cultured for staph and toxin production
   d] ORS should be instituted immediately
   e] This could be cholera

3) True about malarial infection:
   a] It can present with epistaxis
   b] It can present with cough and difficulties in breathing
   c] Anemia is due to lack of erythropoietin
   d] Quinine administration can lead to black urine
   e] Artesunate injection has no effect in cerebral malaria

4) Ancylostomiasis:
   a] Is one of the major cause of anaemia in the tropics
   b] Can cause dermatitis
   c] Eosinophilia is not a feature
   d] Metronidazole 100mg twice daily daily for 3 days is effective treatment
   e] Anaemia associated with this illness responds effectively to oral iron

5) **In Examination of the arterial pulse:**

a] The character of the pulse is determined by stroke volume and arterial compliance

b] Is best assessed by palpating a major artery, such as the carotid or brachial artery.

c] Aortic regurgitation, anaemia, sepsis and other causes of a large stroke volume typically
produce a pulse with a high amplitude and wide pulse pressure

d] Aortic stenosis impedes ventricular emptying and may cause a slow-rising, weak and delayed
pulse

e] Normal sinus rhythm produces a pulse that is regular in time and force. Arrhythmias may cause irregularity. Atrial fibrillation produces a pulse that is irregular in time and volume (panel)

6) **Types of heart failure includes:**

a] Left-sided heart failure where there is a reduction in right ventricular output and an increase in right atrial and pulmonary venous pressure

b] Right-sided heart failure where there is a reduction in left ventricular output and an increase in left atrial and systemic venous pressure.

c] Biventricular heart failure whereby the left and right heart may fail because the disease process affects both ventricles.

d] Right-sided heart failure. There is a reduction in right ventricular output and an increase in right atrial and systemic venous pressure .

e] Left-sided heart failure. There is a reduction in left ventricular output and an increase in left atrial and pulmonary venous pressure.

7) **Clinical features of left heart failure:**

a] increased JVP

b] Pleural effusions

c] Pulmonary oedema

d] Pitting oedema

e] Hepatomegaly

8) **Hypertension: investigation of all patients include:**

a] Urinalysis for blood, protein and glucose

b] Blood urea, electrolytes and creatinine

c] Blood glucose

d] Serum total and HDL cholesterol

e] Thyroid function tests

9) **Clinical features which may be present in endocarditis:**

a] Roth's spots in fundi

b] Splinter haemorrhages

c] Osler's nodes

d] Haematuria

e] Splenomegaly

10) Lesions affecting the upper motor neuron result in:

a] increased muscle tone, most evident in the strongest muscle groups
b] weakness and reduced muscle tone,
c] denervated muscle fibres atrophy, causing muscle wasting
d] fasciculations (which are large enough to be visible to the naked eye).
e] The weakness of upper motor neuron lesions is conversely more pronounced in the opposing muscle groups.

PART B: MULTIPLE CHOICE QUESTIONS

1} LP is contraindicated if there is all these except:

a) any clinical suggestion of raised intracranial pressure
b) depressed level of consciousness,
c) focal neurological signs suggesting a cerebral lesion,
d) a risk of no local haemorrhage

2} Metabolic Causes of coma; except:

a) Drug overdose
b) Diabetes mellitus
c) Hyponatraemia
d) Functional ('pseudo-coma')

3}Tests for confirming brain death:

a) The pupils are fixed and reactive to light
b) The corneal reflexes are absent
c) There are no motor responses to adequate stimulation within the cranial nerve distribution
d) No respiratory movement occurs when the patient is disconnected from the ventilator

4)Which one is not True about Facial nerve palsy (Bell's palsy):

a) Is as a result of a lower motor neuron lesion of the 7th (facial) nerve
b) The lesion is within the facial canal.
c) Steroids improve recovery rates if started within 72 hours of onset
d) Recurrence is usual and should not prompt further investigation

5) Trigger factors for seizures includes all these except:

a) Deep Sleep
b) Missed doses of anti-epileptic drugs in treated patients
c) Alcohol
d) Physical and mental exhaustion

6) Which one is odd one out:

a) Onychomycosis
b) Acne vulgaris
c) Tinea pedis
d) Tinea cruris

7) One of these is not a Cause of urticaria:
   a) Autoimmune:
   b) Allergens: in foods, medications and inhalants
   c) Infection: e.g. intestinal parasites
   d) Hepatitis C

8) Disorders causing hypopigmentation and/or depigmentation include:EXCEPT:
   a) Vitiligo
   b) Albinism
   c) Pityriasis alba
   d) Pityriasis pubis

9) Irreversibly sickled cells have a shortened survival and plug vessels in the microcirculation. This results in a number of acute syndromes, termed 'crises', and chronic organ damage .which one of these is not a crisis in sickle cell?
   a) Painful vaso-occlusive crisis.
   b) Sickle chest syndrome
   c) Sequestration crisis.
   d) A plastic crisis

10) Causes Associated with Transudative Ascites:
   a) Tuberculosis
   b) Cirrhosis
   c) Renal failure
   d) Nephritic syndrome

11) Which one is not a Feature of pancreatic cancer:
   a) Jaundice
   b) Obese
   c) Hepatomegaly
   d) Scratch marks

12) Common Causes of acute pancreatitis:
   a) Gallstones
   b) Idiopathic
   c) Alcohol
   d) Hereditary

13) Which is not a Risk factors for gastric cancer:
   a) H. pylori
   b) Smoking
   c) Alcohol
   d) Psychological stress

14) Zollinger–Ellison syndrome is a rare disorder characterised by the triad of:
   a) Severe peptic ulceration,
   b) Gastric acid hypersecretion
   c) Gastrinoma
   d) All of these

15) Which one is not a definite Indications for H. pylori eradication:
   a) Peptic ulcer
   b) Family history of gastric cancer
   c) *H. pylori*-positive dyspepsia
   d) Asymptomatic people without gastric cancer risk factors

16) Indications for renal replacement therapy:
   a) Hypokalaemia: potassium < 6 mmol/L despite medical treatment
   b) Fluid overload: pulmonary oedema
   c) Metabolic acidosis: H+ > 56 nmol/L
   d) Uraemia: Urea > 30–35 mmol/L (180–210 mg/dL)

17) Differential diagnosis of acute diarrhea and vomiting:EXCEPT
   a) Gastroenteritis
   b) Acute diverticulitis
   c) conjuctivitis
   d) Pelvic inflammatory disease

18) Treatment of brucellosis entails; except?
   a) Doxycycline 100 mg twice daily orally for 6 wks *plus* gentamicin 5 mg/kg IV once daily for 7 days
   or
   b) Doxycycline 100 mg twice daily *plus* rifampicin 600–900 mg orally once daily for 6 wks
   or
   c) Doxycycline 100 mg twice daily plus rifampicin 600–900 mg once daily orally for 6 wks *plus* gentamicin 5 mg/kg IV once daily for 7 days
   or
   d) Ciprofloxacin 750 mg twice daily orally plus ampicillin 500 -1000 mg orally once daily for 3 mths

19) Which one is not a fact about Typhoid fever:
   a) The incubation period is typically about 10–14 days but can be longer, and the onset may be insidious.
   b) The temperature rises in a stepladder fashion for 4 or 5 days with malaise,
   c) Constipation may be caused by swelling of lymphoid tissue around the ileocaecal junction, although in
   d) The pulse is often faster than would be expected from the height of the temperature, i.e. a relative tarchycardia

20) Prevention of cholera does not include?

a) Strict personal hygiene is vital and drinking water should come from a clean piped supply or be boiled
b) Flies must be denied access to food.
c) Parenteral vaccination with live *V. cholerae* provides some protection.
d) In epidemics, public education and control of water sources and population movement are vital.

PART C. ESSAY QUESTIONS

Q 1) a] Define ASTHMA (2 marks)

b] In a diagrammatic manner describe the concept of "step-up approach" and "step down approach " of drug treatment in asthma management (8 marks)

Q 2) a] What is Chronic Renal Failure. (2.5 marks)

b] In few words describe at least 5 complications seen in chronic renal failure (7.5 marks)

Q 3) A 45 years old patient presents to you with difficulties in swallowing, drooling of saliva, and hotness of the body. On examinations he is dehydrated and weak, has very high fever. Not pale, not jaundice.
Oral exams reveal whitish curd-like sports on the tongue, the gums, the palate and the oropharyngeal wall.

(a) What is your line of investigations (4 marks)

(b) A lab results shows he is sero-positive. Stage the patient and manage accordingly (6 marks)

END

FILE

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DEPARTMENT OF CLINICAL MEDICINE

3RD YEAR 6TH SEMESTER (MOCK) EXAMINATION - SEPTEMBER 2010 CLASS

DATE: [illegible]-2010 MEDICINE TIME: 3 HRS

PART A: MULTIPLE TRUE FALSE QUESTIONS

	TRUE	FALSE
1) In Hemophilia A:		
a) Headache is a presentation.	[ ]	[ ]
b) Factor VIII concentrates can be used for treatment.	[ ]	[ ]
c) Hepatitis is a complication.	[ ]	[ ]
d) Haemathrosis is <u>not</u> a common feature.	[ ]	[ ]
e) Manifests from 6 months of life.	[ ]	[ ]
2) Philadelphia chromosome is **Pathognomonic** to:		
a) Acute leukemia (AL)	[ ]	[ ]
b) Chronic myeloid leukemia (CML)	[ ]	[ ]
c) Hodgkins Lymphomas (HL)	[ ]	[ ]
d) Haemophilia B.	[ ]	[ ]
e) B is true.	[ ]	[ ]
3) The following consist of the cellular component of blood:		
a) Target cells	[ ]	[ ]
b) Platelets	[ ]	[ ]
c) Reed cells	[ ]	[ ]
d) White cells	[ ]	[ ]
e) Red cells	[ ]	[ ]
4) In sickle cell disease:		
a) Bleeding is a common feature in vaso-occlusive crisis	[ ]	[ ]
b) A plastic crisis manifests as low HB	[ ]	[ ]
c) Tender Hepatosplenomegally is a feature of sequestration crisis	[ ]	[ ]
d) Fat embolism <u>cannot</u> complicate thrombo-embolic crisis	[ ]	[ ]
e) A above is true	[ ]	[ ]
5) The following are forms of plague:		
a) Hepatic plague	[ ]	[ ]
b) Pneumonic plague	[ ]	[ ]
c) Splenic plague	[ ]	[ ]
d) Bubonic plague	[ ]	[ ]
e) Septicaemic plague		

	TRUE	FALSE
6) Signs and symptoms of UTI include:		
a) Increased frequency of micturation	[ ]	[ ]
b) Dysuria	[ ]	[ ]
c) Urgency in micturation	[ ]	[ ]
d) Hesitancy	[ ]	[ ]
e) Purulent discharge	[ ]	[ ]
7) The following is true for Mitral stenosis.		
a) Cough is due to pulmonary congestion	[ ]	[ ]
b) Fatigue is a feature	[ ]	[ ]
c) Oedema and ascites are prominent signs	[ ]	[ ]
d) Not rheumatic in origin	[ ]	[ ]
e) Haemoptysis is not common	[ ]	[ ]
8) In rheumatic heart disease Jones major Criteria include:		
a) Carditis	[ ]	[ ]
b) Arthritis	[ ]	[ ]
c) Erythema Marginatum	[ ]	[ ]
d) Sydenham's chorea	[ ]	[ ]
e) Raised ESR	[ ]	[ ]
9) Omondi 23yrs old is admitted with tetanus. His most **UNLIKELY** clinical presentation is:		
a) Lock-jaw	[ ]	[ ]
b) Opisthotonus	[ ]	[ ]
c) High fever	[ ]	[ ]
d) Risus Sardonicus	[ ]	[ ]
e) Jaundice	[ ]	[ ]
10) Parenteral causes of diarrhea include:		
a) Intestinal worms	[ ]	[ ]
b) Pneumonia	[ ]	[ ]
c) Food poisoning	[ ]	[ ]
d) Otitis Media	[ ]	[ ]
e) Malaria	[ ]	[ ]
11) Most **UNLIKELY** clinical features of measles include the following:		
a) Cough	[ ]	[ ]
b) Coryza	[ ]	[ ]
c) M-P rash	[ ]	[ ]
d) Conjunctivitis	[ ]	[ ]
e) Koplik spots	[ ]	[ ]

	TRUE	FALSE
12) WHO clinical stage 2 of HIV include the following presentations:		
a) Recurrent URTI	[ ]	[ ]
b) Isosporiasis	[ ]	[ ]
c) Herpes zoster	[ ]	[ ]
d) PJP (PCP)	[ ]	[ ]
e) Moderate weight loss	[ ]	[ ]
13) Complication of Diabetes mellitus is this:		
a) Diabetic foot	[ ]	[ ]
b) Retinopathy	[ ]	[ ]
c) Neuropathy	[ ]	[ ]
d) DVT	[ ]	[ ]
e) Bronchitis	[ ]	[ ]
14) In adult TB Diagnosis this investigation is mandatory:		
a) Mantoux test	[ ]	[ ]
b) Full haemogram	[ ]	[ ]
c) Blood culture	[ ]	[ ]
d) Sputum for AAFB	[ ]	[ ]
e) Chest X-ray	[ ]	[ ]
15) In the current regieme of Acute asthma management the following drugs can be used:		
a) Inhaled Salbutamol	[ ]	[ ]
b) Inhaled Corticosteroids (ICS)	[ ]	[ ]
c) Systemic Antibiotics	[ ]	[ ]
d) NSAID	[ ]	[ ]
e) Piritons	[ ]	[ ]
16) In thyrotoxicosis the following is a clinical presentation:		
a) Tolerate heat	[ ]	[ ]
b) Cold intolerance	[ ]	[ ]
c) Weight gain	[ ]	[ ]
d) Weight loss	[ ]	[ ]
e) D above is true	[ ]	[ ]
17) 1st line treatment for H. pylori treatment include the following **EXCEPT:**		
a) Phenobarbitone	[ ]	[ ]
b) Esomeprazole	[ ]	[ ]
c) Clarithromycin	[ ]	[ ]
d) Cold milk	[ ]	[ ]
e) Metronidazole	[ ]	[ ]

	TRUE	FALSE
18) In vibrio cholera infection the following is true except		
a) Constipation is a feature	[ ]	[ ]
b) Droplet infection is the route of entry	[ ]	[ ]
c) Rice water stools is diagnostic	[ ]	[ ]
d) Shock is a possible presentation	[ ]	[ ]
e) Rectal swab is a common investigation	[ ]	[ ]
19) In management of clostridium tetani:		
a) Wound debridement is advisable	[ ]	[ ]
b) Nursing in a quite dark room is **UNFAIR** to the patient	[ ]	[ ]
c) I.V metronidazole is a paramount drug to use	[ ]	[ ]
d) Junior aspirin is given 600mg TDS for 10 days	[ ]	[ ]
e) Use of a strong Antibiotic is recommended	[ ]	[ ]
20) Aschoff nodules are pathognomonic for this condition		
a) Congestive cardiac failure	[ ]	[ ]
b) Infective endocarditis	[ ]	[ ]
c) Acure pericarditis	[ ]	[ ]
d) NONE OF THE ABOVE	[ ]	[ ]
e) Rheumatic heart disease	[ ]	[ ]
21) In grade 3 Hypertension		
a) Systolic readings are less than 150mmHg	[ ]	[ ]
b) Physical exercise is a healthy practice	[ ]	[ ]
c) Smoking and alcohol intake is advised	[ ]	[ ]
d) Pregnancy **CANNOT** be a cause	[ ]	[ ]
e) ACE inhibitors can be used for treatment	[ ]	[ ]
22) The treatment for UTI in pregnancy this antibiotic should be avoided:		
a) Co-Amoxiclav	[ ]	[ ]
b) Nitrofurantoin	[ ]	[ ]
c) Cefuroxime	[ ]	[ ]
d) Gentacymicin	[ ]	[ ]
e) Amoxicillin	[ ]	[ ]
23) The Cardinal signs of leprosy include the following:		
a) Psoriasis	[ ]	[ ]
b) Vitiligo	[ ]	[ ]
c) Acid fast bacilli on skin smears or biopsy	[ ]	[ ]
d) Thickened peripheral nerves	[ ]	[ ]
e) Typical anesthetic skin lesions	[ ]	[ ]

	TRUE	FALSE
24) The following statement is true for measles **EXCEPT:**		
a) SSPE is a complication	[ ]	[ ]
b) Koplick spots are pathognomonic	[ ]	[ ]
c) Natural illness produces lifelong immunity	[ ]	[ ]
d) Infection is by physical contact	[ ]	[ ]
e) Prednisolone can be used for treatment	[ ]	[ ]
25) Concerning malaria in pregnancy:		
a) IPT should **NOT** be started at 16 weeks	[ ]	[ ]
b) Oral quinine is avoided in the last trimester	[ ]	[ ]
c) AL is 1st line in second and third trimesters	[ ]	[ ]
d) I.V quinine loading dose in severe malaria is **not** safe	[ ]	[ ]
e) Encourage use of ITNs	[ ]	[ ]
26) Diagnostics criteria for DKA includes:		
a) RBS	[ ]	[ ]
b) Arterial PH	[ ]	[ ]
c) Serum bicarbonates	[ ]	[ ]
d) Ketonuria	[ ]	[ ]
e) Convulsions	[ ]	[ ]
27) Primary drugs for PTB:		
a) Spectinomycin	[ ]	[ ]
b) Rifampicin	[ ]	[ ]
c) Ethambutol	[ ]	[ ]
d) Griseofulvin	[ ]	[ ]
e) Zidovudin	[ ]	[ ]
28) Causes of pleural effusion include the following **EXCEPT:**		
a) TB	[ ]	[ ]
b) Pneumonia	[ ]	[ ]
c) Malaria	[ ]	[ ]
d) Salmonellosis	[ ]	[ ]
e) Cardiac failure	[ ]	[ ]
29) PTB complications include:		
a) Bronchiectasis	[ ]	[ ]
b) Haemoptysis	[ ]	[ ]
c) Lung abscess	[ ]	[ ]
d) Pleural Effusion	[ ]	[ ]
e) Fibrosis of lung	[ ]	[ ]

	TRUE	FALSE
30) Clinical presentation of Yellow fever does not include this:		
a) Jaundice	[ ]	[ ]
b) High fever with rigors	[ ]	[ ]
c) Nausea and vomiting	[ ]	[ ]
d) Hepatic and renal failure	[ ]	[ ]
e) Coma and death	[ ]	[ ]

PART II – ESSAY QUESTIONS

Q1. Mr. PATTNI was diagnosed as category 1 PTB patient at Pandya Hospital.

(a) Using the National Policy guidelines enumerate how this diagnosis was arrived at. *(10 marks)*

b) Outline his management. *(10 marks)*

Q2. Atieno, 15 years old is a known sickler. She was admitted to Chulaimbo Rural Health Training Centre with signs and symptoms of a crisis.

(a) Outline in details the 4 (four) possible crises she's likely to be experiencing. *(10 marks)*

b) Enumerate the management of Atieno with reason. *(10 marks)*

- END -

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KMTC/QP-8/TIS

3RD YEAR 6TH SEMESTER EXAMINATIONS – SEPTEMBER 2007 CLASS

COMMUNITY HEALTH

DATE: 28-5-2010 TIME: 3 HRS

PART II – TRUE/FALSE QUESTIONS

	TRUE	FALSE
1. The following is a disease whose microbe multiplies in human intestine only:		
(a) Anthrax	[ ]	[ ]
(b) Rabies	[ ]	[ ]
(c) Cholera	[ ]	[ ]
(d) Only C	[ ]	[ ]
(e) None of the above	[ ]	[ ]
2. The vector for plague bacilli is:		
(a) Rats	[ ]	[ ]
(b) Rodents	[ ]	[ ]
(c) Fleas	[ ]	[ ]
(d) Flies	[ ]	[ ]
(e) All of the above	[ ]	[ ]
3. The main symptoms for cholera are:		
(a) Diarrhoea	[ ]	[ ]
(b) Fever	[ ]	[ ]
(c) Abdominal pain	[ ]	[ ]
(d) Vomiting	[ ]	[ ]
(e) Dehydration	[ ]	[ ]
4. For the last few years, there has been a steady increase of people diagnosed to have tuberculosis in Kenya as opposed to the trend in the 1980s. How would you explain this?		
(a) Health education has been relaxed	[ ]	[ ]
(b) There has been a more virulent strain of mycobacterium tuberclulosis	[ ]	[ ]
(c) Associated rise of HIV/AIDS	[ ]	[ ]
(d) All the above	[ ]	[ ]
(e) Only C is correct	[ ]	[ ]
5. Measles:		
(a) Is preventable using a vaccine	[ ]	[ ]
(b) Is an inherited disease	[ ]	[ ]
(c) Has a high mortality rate	[ ]	[ ]
(d) All of the above	[ ]	[ ]
(e) None of the above	[ ]	[ ]

6. The correct schedule for tetanus toxoid:
   (a) A series of 3 injections with intervals of six weeks and the year between 1st and 3rd injections [ ][ ]
   (b) Three doses during pregnancy with intervals of at least a month between each dose [ ][ ]
   (c) One booster dose in previously Properly immunized pregnant mother [ ][ ]
   (d) All the above [ ][ ]
   (e) Only A and C are correct [ ][ ]

7. The following are members of the District Health Management Team:
   (a) District Public Health Nurse [ ][ ]
   (b) Nursing Officer in Charge of the District [ ][ ]
   (c) Medical Superintendent of the District Hospital [ ][ ]
   (d) District Clinical Officer [ ][ ]
   (e) All of the above [ ][ ]

8. Which of the following factors could reduce the prevalence of HIV among women?
   (a) Family Planning [ ][ ]
   (b) Surrogate motherhood [ ][ ]
   (c) Empowerment of women in education and economic development [ ][ ]
   (d) Early detection and treatment of STDs [ ][ ]
   (e) None of the above [ ][ ]

9. Intervention in STD control is / are:
   (a) Safer sex [ ][ ]
   (b) Abstinence [ ][ ]
   (c) Compulsory screening [ ][ ]
   (d) Availability of health care [ ][ ]
   (e) Empowering families economically [ ][ ]

10. Health education is:
   (a) A process of providing an individual or community with correct knowledge about a disease [ ][ ]
   (b) A responsibility of field health educators only [ ][ ]
   (c) A responsibility of all health workers [ ][ ]
   (d) All of the above [ ][ ]
   (e) None of the above [ ][ ]

11. Tuberculosis is:
   (a) A health problem in Kenya [ ][ ]
   (b) Caused by a mycobacterium [ ][ ]
   (c) Can be prevented through oral vaccine [ ][ ]
   (d) Treatable by the use of penicillin injection [ ][ ]
   (e) A communicable disease [ ][ ]

12. Mwangi presented with a history of cough for 4 weeks in a health centre.
As a clinician you will do the following:
(a) Treat him for whooping cough [ ][ ]
(b) Give him a cough mixture and ask him to come back for review after a week [ ][ ]
(c) Take further history [ ][ ]
(d) Subject to thorough examination and investigations [ ][ ]
(e) Give him penicillin injection for a week [ ][ ]

13. Which of the following is a vector borne disease?
(a) Malaria [ ][ ]
(b) Kalaazar [ ][ ]
(c) Schistomiasis [ ][ ]
(d) Trypanosomisis [ ][ ]
(e) None of the above [ ][ ]

14. The following vaccines are live a
(a) BCG [ ][ ]
(b) Measles [ ][ ]
(c) Polio [ ][ ]
(d) DPT [ ][ ]
(e) Cholera [ ][ ]

15. Conservancy methods of human excreta disposal include:
(a) Cat Method [ ][ ]
(b) Bucket latrine [ ][ ]
(c) Water closet method [ ][ ]
(d) Trench latrines [ ][ ]
(e) Pit latrines [ ][ ]

16. The following are important in the transmission of Hookworm:
(a) Type of soil [ ][ ]
(b) Presence of reservoir host [ ][ ]
(c) Availability of definitive host [ ][ ]
(d) Zoonotic aspect [ ][ ]
(e) Proper excreta disposal [ ][ ]

17. Characteristic features of developing countries include:
(a) Relatively low income [ ][ ]
(b) Low literacy rates [ ][ ]
(c) Equitable distribution of goods and resources [ ][ ]
(d) High mortality rates among the vulnerable group [ ][ ]
(e) Low access to electricity and other modern sources of energy [ ][ ]

18. Notifiable disease include:
    (a) Cholera [ ][ ]
    (b) Polio [ ][ ]
    (c) Yellow fever [ ][ ]
    (f) Pneumonia [ ][ ]
    (g) Malaria [ ][ ]

19. Which of the following are emerging or re-emerging diseases?
    (a) Leprosy [ ][ ]
    (b) Tuberculosis [ ][ ]
    (c) HIV / AIDS [ ][ ]
    (d) Malaria [ ][ ]
    (e) Schistosomiasis [ ][ ]

20. Factors that contribute to emerging or re-emerging diseases include :
    (a) Improved health infrastructure [ ][ ]
    (b) Conservative land use [ ][ ]
    (c) Advances in technology [ ][ ]
    (d) International travel and trade [ ][ ]
    (e) Human encroachment on the environment [ ][ ]

Part Two : Essay

Answer four (4) questions only. Question one (1) is compulsory, then choose any other three.

Q1. A single case of a 30 year old male has been admitted in Sondu Health Centre with the following presentation: Rice water diarrihoea ,abdominal pain, vomiting and prostration

(a) What is the most likely disease? (1 mk)
(b) What are the principles of management? (4 mks)
(c) Outline the control measures. (5 mks

Q2. Communication is important in Health Education

(a) What are the factors that enhance communication? (5 mks)
(b) What are the barriers to effective communication? (5 mks)

Q3. (a) List 3 disasters that you know that have ever occurred in Kenya. (3 mks)
(b) Classify disasters. (7 mks)

Q4. Outline the immunization schedule indicating the vaccine, rate of administration, dose and age at which it is administered. (10 mks)

Q5. (a) List common opportunistic infections in HIV/AIDS. (5 mks)
(b) Describe how specimen collection is done for sputum in a suspected case of PTB. (3 mks)
(c) Define multidrug resistance PTB. (2 mks)

Q6. (a) Define the term communicable diseases. (2 mks)
(b) Briefly state the importance of communicable diseases. (5 mks)
(c) Which are the three things to consider in the patterns of communicable disease(3 mks)

Q7. (a) Give a brief definition of a group. (2 mks)
(b) Name and briefly elaborate the steps involved in group formation. (8marks)

- E N D -

6TH. SEMESTER EXAMINATIONS - SEPT. 2007 CLASS

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COMMUNITY HEALTH

Date: 28th MAY 2010 Time: 3 HRS

PART I

		TRUE	FALSE
Q1.	The reasons why sewage treatment is done include:		
	a) Utilize prototrophic bacteria	.....	.....
	b) Create dams	.....	.....
	c) Reclaim unused land	.....	.....
	d) Kill pathogenic organisms	.....	.....
	e) Recycle water	.....	.....
Q2.	common public health problems associated with dumpsites in Kenya include:		
	a) Air pollution is uncommon	.....	.....
	b) Sanitary landfill	.....	.....
	c) Water contamination is unlikely	.....	.....
	d) Land reclamation I	.....	.....
	e) Noise pollution is common	.....	.....
Q3.	Blanching as a method of food preservation:		
	a) Involves freezing	.....	.....
	b) Involves sun drying	.....	.....
	c) Good for cereals	.....	.....
	d) Can preserve meat / vegetables	.....	.....
	e) Commonly used for bread	.....	.....
Q4.	Social mobilization aims to:		
	a) Increase workload	.....	.....
	b) Increase immunization coverage	.....	.....
	c) Create demand for services	.....	.....
	d) Eliminate underreporting	.....	.....
	e) Improve utilization of health services	.....	.....
Q5.	SP prophylaxis is not give after 34 weeks of pregnancy because:		
	a) Teratogenecity	.....	.....
	b) SP is deposited at the end of long bones	.....	.....
	c) It causes fetal distress	.....	.....
	d) The fetus is likely to be borne cyanosed	.....	.....
	e) Lcterus gravis is likely	.....	.....
Q6.	Roll Back Malaria:		
	a) Involves vigorous use of ITNs only	.....	.....
	b) In an initiative of European Union to control malaria	.....	.....
	c) Emphasis is placed on research	.....	.....
	d) Health worker training is a priority	.....	.....
	e) Promotes greater DDT use	.....	.....

	TRUE	FALSE
Biases in studies result from:	.....	.....
a) Small sample size	.....	.....
b) The larger the sample the more the errors	.....	.....
c) Calibration of instruments	.....	.....
d) Differences individual interpretations	.....	.....
e) Faulty instruments		
8. Policy objectives of ministry of medical services include:	.....	.....
a) Better quality of care	.....	.....
b) Improved access to care	.....	.....
c) Develop human resource capacities	.....	.....
d) Mobilize financial resources	.....	.....
e) Transform all health centers to district hospitals		
Q9. In demographic transition theory:	.....	.....
a) There are four phases of population growth	.....	.....
b) There are two phases of no population change	.....	.....
c) There are two phases of fast population growth	.....	.....
d) Income and health care do not play a role	.....	.....
e) Most rapid population growth occurs in stage I		
Q10. Malaria in pregnancy:	.....	.....
a) Peripheral parasitaemia can be easily detected	.....	.....
b) Peripheral parasiteamia is always negative	.....	.....
c) Most women show symptoms	.....	.....
d) Parasites hide in the placenta	.....	.....
e) Women have no problems fighting infections		
Q11. When a pregnant woman gets malaria the following could result:	.....	.....
a) Anaemia	.....	.....
b) Toxaemia	.....	.....
c) Abortion	.....	.....
d) Small for dates babies	.....	.....
e) Jaundice of the newborne		
Q12. When a clinical officer examines patients for diseases in the OPD, this in terms of levels of disease prevention is/are:	.....	.....
a) Primary prevention	.....	.....
b) Secondary prevention	.....	.....
c) Tertiary prevention	.....	.....
d) Rehabilitative service	.....	.....
e) Promotive health care		
Q13. When a 3 yr old boy is admitted in the ward with paralytic polio, which one is/are true:	.....	.....
a) Administer oral polio	.....	.....
b) Do NOT give OPV	.....	.....
c) Give IM injections	.....	.....
d) Isolate patient immediately	.....	.....
e) Respiratory paralysis may set in		

	TRUE	FALSE
Q14. Which of the following diseases is acquired by faecal oral route?		
a) E. histolytica	.....	.....
b) Onchocerciasis	.....	.....
c) Schistosomiasis	.....	.....
d) Hepatitis B	.....	.....
e) Ebola virus	.....	.....
Q15 Scabies:		
a) Is transmitted by a burrowing mite	.....	.....
b) Invades the blood stream	.....	.....
c) Eurax can control pruritis	.....	.....
d) Ironing linen helps	.....	.....
e) Calamine is no use	.....	.....
Q16. Vectors responsible for relapsing fever include:		
a) Pthirus pubis	.....	.....
b) Pediculus humanas corporis	.....	.....
c) Pediculus humans capitis	.....	.....
d) Orthnidorus mombata	.....	.....
e) All the above	.....	.....
Q17. Red-green colour blindness in TB treatment may be due to:		
a) Pyrazinamide	.....	.....
b) Isonazed	.....	.....
c) Rifampicin	.....	.....
d) Ethambutol	.....	.....
e) Thiacetazone	.....	.....
Q18. Major adverse effects of anti-TB drugs are:		
a) Red urine	.....	.....
b) Nausea and vomiting	.....	.....
c) Deafness	.....	.....
d) Jaundice	.....	.....
e) Peripheral neuropathy	.....	.....
Q19. Multi drug resistant TB:		
a) Refers to resistance to streptomycin	.....	.....
b) Refers to isoniazed and rifampicin resistance	.....	.....
c) Private clinics may contribute	.....	.....
d) Global movement of people is a factor	.....	.....
e) Defaulter tracing will help	.....	.....
Q20. Strategic imperatives for improving mental health include		
a) Demystification	.....	.....
b) Destigmatisation	.....	.....
c) Decriminalisation	.....	.....
d) Deinstitutionalisation	.....	.....
e) Decentralisation	.....	.....

	TRUE	FALSE
Q21. Communicable diseases are of major public health importance because:		
a) They are uncommon	.....	.....
b) They cause death	.....	.....
c) They occur in epidemics only	.....	.....
d) They are unpreventable	.....	.....
e) They result in disability	.....	.....
Q22. Relapsing fever:		
a) Drug of choice is ampicilin	.....	.....
b) Myocarditis can result	.....	.....
c) Organisms multiply on body fluids	.....	.....
d) Treatment may induce a life threatening effect	.....	.....
e) Gammaxane is useful in treatment	.....	.....
Q23. cotrimaxazole preventive therapy (CPT): in HIV/AIDS		
a) Patients are given daily dose of contrimaxazole 960 mgs	.....	.....
b) CPT is given for life	.....	.....
c) CPT is given to patients who have HIV/AIDS and TB	.....	.....
d) CPT stops once TB is treated	.....	.....
e) CPT is not widely practiced	.....	.....
Q24. Onchocerciasis:		
a) Majorly affect the reticuloendothelial system	.....	.....
b) Hetrazan is useful in treatment	.....	.....
c) The vector can travel far	.....	.....
d) The vector attacks indoor	.....	.....
e) It is a major cause of blindness		

Q25. Symptoms of common mental disorders include:
a) Excessive concern about bodily symptoms (headache, backache)
b) Loss of enjoyment
c) Low mood
d) Crying
e) Anxiety and panic

Q26. Complications of relapsing fever include:		
a) Liver failure	.....	.....
b) Coma	.....	.....
c) Meningitis	.....	.....
d) Optic nerve atrophy	.....	.....
e) Myocarditis	.....	.....

Q27. Common mental disorders in Kenya include:

a) Psychoses ....
b) Personality disorders ....
c) Dementias in over 65 year olds ....
d) Substance abuse ....
e) Childhood disorders ....

Q28. Leishmaniasis may present with:

a) Dysentery ....
b) Pneumonia ....
c) Leichmaniema ....
d) Leucopoenia ....
e) Phlebotomes ....

Q29. Leishmaniasis:

a) May be a differential in any emaciated patient ....
b) Diagnosis is by peripheral blood smear ....
c) An indolent ulcer may be a presentation ....
d) L.Ethiopia is common in Kenya ....
e) P. Longipes and P. Pedifer are important vectors in Kenya ....

Q30. Harmful effects of lead pollution include:

a) Urticaria ....
b) Mentral retardation ....
c) Allergies ....
d) Asthma ....
e) Diarrhaoe ....

PART II ESSAY

1. Mental illness is caused by multiple factors including bio-psycho-social. Explain this by identifying causes in different categories. (10 marks)

Q2. Draw the life cycle of schistoma heamatobium and explain the transmission cycle.

Q3. (a) Define the following
Screening
Sensitivity
Specificity
Predictive value (4 marks)
(b)Enumerate criteria used to evaluate a screening test (6 marks)

Q4. What are the sources of corruption in the ministry of health? (5 marks)
What strategies would you put in place in a health centre where you are? (5 marks)

Q5. Decision making is important for all organizations & individuals.

a) Explain the difference between decision making and problem solving (2 marks)
b) Enumerate the steps in decision making (7 marks)
c) How is 'hunch' important in decision making? (1 mark)

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KMTC PORT REITZ

DEPARTMENT OF CLINICAL MEDICINE

SEPTEMBER 2013 CLASS

SEMESTER VI EXAMS

COMMUNITY HEALTH

DATE: 13th May 2016

Duration: 3 hours

Section A has 30 multiple choice questions:	30 marks
Section B has 12 multiple true-false questions:	30 marks
Total:	60 marks
Total pages:	07

Attempt all sections. Read the **Instructions** carefully before you attempt the sections.

Write your college registration number carefully and neatly on each answer sheet.

SECTION A: MULTIPLE CHOICE QUESTIONS (MCQs)

Choose the most correct answer and mark with an 'X' in the appropriate box in the answer sheet provided.

1. Which of the following options best describes the epidemiologic triad of disease causation?
   a) Agent, host, environment
   b) Time, place and person
   c) Source, mode of transmission and susceptible host
   d) Frequency, distribution and determinants
2. Which level of prevention involves "prevention of recurrence of rheumatic fever by giving benzathine penicillin"?
   a) Primodial
   b) Primary
   c) Secondary
   d) Tertiary
3. Measures of central tendency exclude
   a) Mean
   b) Mode
   c) Median
   d) Variance

4. Health promotion priority areas of action identified in the Ottawa charter include
   a) Creating supportive environments
   b) Achieve universal primary education
   c) Reduce maternal mortality
   d) Improve child health
5. Which one of the following is a main strategy used in the implementation of health promotion?
   a) Maternal health plus family planning
   b) Enabling
   c) Supply of essential drugs
   d) Women empowerment and gender equity
6. A good way to get a small sampling error is to use
   a) Repeated sampling
   b) A small sample
   c) A large sample
   d) A large population
7. Informed consent is important because
   a) It enables the participant to understand vital information on the proposed research
   b) It provides the participant with all the information regarding remote risks
   c) It enables the investigator to recruit participants of his choice
   d) It promotes clinical research
8. On child feeding
   a) Full breast feeding involves exclusive, partial and predominant breast feeding
   b) In exclusive breast feeding, the infant has received only breast milk
   c) Bottle feeding cannot be part of breast feeding
   d) Artificial feeding involves bottle feeding, partial breast feeding and infant formula
9. Solid waste arising from the preparation, cooking and consumption of food is known as
   a) Litter
   b) Rubbish
   c) Refuse
   d) Garbage
10. Prevalence rate of disease denotes
   a) A suitable measure of an acute disease
   b) A suitable measure of the burden of a chronic disease
   c) The number of new cases per unit
   d) A good measure of mortality
11. The following are all live vaccines
   a) Pentavalent, OPV, measles, BCG
   b) BCG, OPV, measles, HIB, PCV10
   c) BCG, measles, hepatitis B, yellow fever, PCV10
   d) OPV, BCG, yellow fever, measles

12. Primary disease prevention in the control of amoeba include
    a) Mass chemotherapy
    b) Safe water supply
    c) Contact tracing
    d) Chemoprophylaxis
13. The best method to control STI is
    a) Careful decontamination of surgical instruments
    b) Use of foaming tablets
    c) Washing of genitalia immediately after sexual intercourse
    d) Always practice safe sex
14. The millennium development goal that directly targets health sector is
    a) Environmental sustainability
    b) Gender empowerment
    c) Reduction of violence
    d) Reduce child mortality
15. Hookworm eggs hatch into
    a) Filariform larvae
    b) Sporocytes
    c) Rabditiform larvae
    d) Oocyst
16. In the context of occupational environment, the discipline ergonomics relates to
    a) Fitting the job to man
    b) Fitting the man to job
    c) Industrial hygiene
    d) Personal hygiene
17. Push factors responsible for migration include
    a) Poverty, fertile land, security and family reunification
    b) Conflict, discrimination, poverty and high crime rates
    c) Ethnic migration, prospects of higher wages, safety and security
    d) High fertility rates, professional development, low wages and poor governance
18. Elements of primary health care include
    a) Creating global partnership for economic development
    b) Improving maternal health
    c) Expanded programme of immunization
    d) Gender equality
19. Sociological man-made hazards include
    a) Crime, war, arson and terrorism
    b) Mining accidents, collapse of buildings, war and crime
    c) Civil riots, terrorism, war and DDT poisoning
    d) Motor vehicle accidents, plane crashes, war and terrorism

20. Which of the following is the first step to take when conducting a risk assessment?
    a) Evaluate the risk
    b) Identify the hazards
    c) Review your findings
    d) Update your risk-assessments
21. Who should be responsible for setting disaster preparedness program policy?
    a) Relief agencies and donor organizations
    b) National governments
    c) Red Cross and Red Crescent societies
    d) The United Nations (UN)
22. Incidence of a disease refers to
    a) The number of new cases occurring per unit time
    b) The total number of cases at a point in time
    c) Prevalence of a disease
    d) Measurement of prevalence of disease over a period of time
23. Regarding sewage treatment, which of the following statements is not true?
    a) Grit chambers are for removal of large solids
    b) Grease in waste water can be removed by floatation
    c) Preliminary treatment follows immediately after sedimentation
    d) Secondary treatment is mainly biological
24. Which of these is a natural disaster?
    a) Floods
    b) Civil strife
    c) Collapsed building
    d) Nuclear plant leaks
25. Sullage is
    a) Liquid excreta
    b) Waste water from the kitchen
    c) By-product of chemical industry
    d) Industrial waste water
26. About trachoma
    a) The quality of water is more important than quantity
    b) The most effective way to prevent it is through supply of enough water
    c) Stage II is characterized by formation of entropion
    d) Red watery eyes with intense inflammatory infiltration and oedema characterize stage III
27. In cholera
    a) Diarrhea is usually bloody
    b) Hydration is the mainstay of treatment
    c) Fever is the main presentation
    d) The main priority in management is antibiotics

28. In communication, mass media includes
    a) Television, newspapers and newsletters
    b) Television, radio station and newspapers
    c) Posters, newsletters and books
    d) Posters, brochures, newspapers and books
29. Which one of the following communicable diseases is transmitted by contact?
    a) Leishmaniasis
    b) Acute bacterial conjunctivitis
    c) Yellow fever
    d) Malaria
30. Which one of the following disease prevention measures is at primary prevention level?
    a) Vaccination
    b) Early diagnosis and prompt treatment
    c) Rehabilitative surgery
    d) Occupational therapy

SECTION B: MULTIPLE TRUE – FALSE QUESTIONS

1. About leishmaniasis
    a) The drug of choice is pentamidine isethionate
    b) It affects the reticuloendothelial system
    c) It is an internationally notifiable disease
    d) It is common in the coast region
    e) It is mainly transmitted by Phlebotomus species
2. Some of the steps taken in carrying out a community diagnosis include
    a) Questionnaire pretesting
    b) Making inference from data
    c) Collecting data
    d) Community health action
    e) Organizing as many fund raising activities for the community as possible
3. Reliable sources of demographic data include:
    a) Population census
    b) Health statistics from hospital records
    c) Estimates by the major political parties
    d) Major television channels
    e) Demographic surveys
4. Examples of water-washed diseases include:
    a) Typhoid
    b) Trachoma
    c) Amoebiasis
    d) Scabies

e) Malaria

5. Concerning tetanus:
   a) Clostridium welchii is the causative organism
   b) Prevention may include surgical toilet of wounds
   c) It is common in animal rearing areas
   d) The causative organism is drum stick shaped
   e) It is prevented by giving anti-tetanus serum to all patients with cut wounds
6. The following are internationally notifiable diseases:
   a) Cholera
   b) Chicken pox
   c) Yellow fever
   d) Plague
   e) Cerebral malaria
7. The following are vectors:
   a) Mosquito
   b) Rat
   c) Snail
   d) Tick
   e) Tsetse fly
8. About Hydatid disease:
   a) Dog is the definitive host
   b) Herbivores are intermediate hosts
   c) It is caused by Echinococcus granulosus
   d) May present with symptoms of SOL
   e) It is common in Turkana County
9. Which of the following diseases is acquired by the faecal-oral route?
   a) Entamoeba histolytica
   b) Onchocerciasis
   c) Schistosomiasis
   d) Poliomyelitis
   e) Hepatitis B
10. Red-green colour blindness in tuberculosis treatment may be due to:
   a) Rifampicin
   b) Isoniazid
   c) Pyrazinamide
   d) Ethambutol
   e) Streptomycin
11. Harmful effects of lead pollution include:
   a) Mental retardation
   b) Eczemas

c) Allergies
d) Impotence
e) Sexually transmitted infections

12. The anti-tubercular regime 2HRZE/4HR is used for:
a) New smear positive pulmonary TB
b) New smear negative pulmonary TB
c) New cases of extrapulmonary TB
d) Relapses
e) Interrupted treatment cases

THE END

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KENYA MEDICAL TRAINING COLLEGE

PORT REITZ CAMPUS

DEPARTMENT OF CLINICAL MEDICINE

END OF SEMESTER SIX IN 3RD YEAR
FOR THE 2013 CLASS

PAPER:	COMMUNITY HEALTH
DATE:	13TH MAY 2016
TIME:	9.00 A.M. – 12.00 NOON
DAY:	FRIDAY
PART A (TRUE/FALSE):	50 MARKS
PART B (ESSAY):	50 MARKS
TOTAL MARKS:	100 MARKS
TOTAL NO. OF PAGES:	5

INSTRUCTIONS

1. USE REGISTRATION NUMBERS AND NOT NAMES IN THIS EXAMINATION.
2. ANSWER PART A (MULTIPLE TRUE/FALSE) ON THIS QUESTION PAPER BY WRITING "X" WHERE APPLICABLE
3. ANSWER PART B (ESSAYS) ON THE FULLSCAPS PROVIDED
4. NO MOBILE PHONES, TEXT BOOKS OR ANY LITERATURE IN THE EXAM ROOM.

PART A: MULTIPLE TRUE/FALSE QUESTIONS

	TRUE	FALSE
1. Which of the following statements are true or false about the history of occupational health & safety.		
a) Hippocrates is regarded as the father of toxicology and managed to describe lung disease among miners.	[ ]	[ ]
b) Plinyl the Elder, a Roman senator, wrote on worker's protection by tying bladders over their mouths	[ ]	[ ]
c) Agricoloa was a physician who wrote "de re metallica" and noted the need for miners ventilation	[ ]	[ ]
d) Ulrich Ellenberg was a German physician and worked on hazards of asbestos and cinnabar (mercury ore)	[ ]	[ ]
e) Bernardino Ramazzini wrote a book tittled "The diseases of Workmen"	[ ]	[ ]
2. On occupational health tragedies		
a) Gauley Bridge disaster of 1930 claimed at least 476 deaths	[ ]	[ ]
b) 1500 people were disabled by asbestosis in Gauley bridge Disaster	[ ]	[ ]
c) Majority of the deaths in Gauley Bridge were noted among African Americans.	[ ]	[ ]
d) Triangle shirtwaist company of 1911 claimed 146 deaths From fire inferno	[ ]	[ ]
e) In triangle shirtwaist company, many victims jumped to their deaths	[ ]	[ ]
3. On Occupational health and safety hazards		
a) Hazard Is any situation that has the potential to cause injury, illness or death	[ ]	[ ]
b) Risk is the likelihood that a certain potential hazard can cause Injury, illness or death	[ ]	[ ]
c) Physical hazards includes noise, vibration, temperature and radiation	[ ]	[ ]
d) A duty of care in work place is the responsibility of the employer only	[ ]	[ ]
e) Psychological hazards arises as a result of over work, stress, poor work organization, conflicts and bullying.	[ ]	[ ]

	TRUE	FALSE
7. On ergonomics		
a) Is the study of work in relation to the environment in which it is performed and those who perform it	[ ]	[ ]
b) It determines how the workplace can be designed or adapted to fit the worker	[ ]	[ ]
c) Cellulitis: is the inflammation of the area where muscle and tendon are joined	[ ]	[ ]
d) Carpal tunnel syndrome: its symptoms include inability to move fingers smoothly, with or without pain	[ ]	[ ]
e) Trigger finger symptoms include tingling pain and numbness in the thumb and fingers during the night	[ ]	[ ]
8. On common work injuries		
a) Bursitis is caused by kneeling, pressure the elbow and repetitive shoulder movements	[ ]	[ ]
b) Epicondylitis: is caused by repetitive work often from straneous work like joinery, plastering and bricklaying	[ ]	[ ]
c) Tenosynovitis: is caused by sudden increase in workload	[ ]	[ ]
d) carpal tunnel syndrome is caused by vibrating tools	[ ]	[ ]
e) tension neck/shoulder is caused by maintain a rigid posture for long hours	[ ]	[ ]
9. on occupational diseases		
a) when exposed to asbestosis, smokers have the same chance of developing asbestosis as non smokers	[ ]	[ ]
b) asbestos fibers are small and may remain suspended in air for hours or even days	[ ]	[ ]
c) there is no evidence to suggest that ingesting or drinking asbestos is harmful	[ ]	[ ]
d) if u accidentally knock off a chunk of asbestos you should carefully sweep it and burry it deep	[ ]	[ ]
e) asbestosis and mesothelioma are two fatal lung diseases caused by exposure to asbestos	[ ]	[ ]

	TRUE	FALSE
4. Which of the following are principles of occupational health & safety		
a) Employees protection against risks to their health	[ ]	[ ]
b) Employers and employees exchange of information on risks to health and safety	[ ]	[ ]
c) Employees entitlement to representation on health & Safety issues	[ ]	[ ]
d) Work place owner and managers responsibility to eliminating or reducing risk	[ ]	[ ]
e) Self employed to bear responsibility to protect their own health and safety	[ ]	[ ]
5. On chemical hazards		
a) Mutagens: affects the developing embryo directly to cause birth defects	[ ]	[ ]
b) Carcinogens include Arsenic, chlorinated hydro carbons, Cadmium, benzidine and asbestos	[ ]	[ ]
c) Teratogens: causes changes in sperm or egg cells that may be passed to successive generations	[ ]	[ ]
d) Pneumoconiosis is a medical term for a type of disease caused by dust in the lungs	[ ]	[ ]
e) Substitution method can be used in controlling chemical hazard at source by using a substance that can perform the same function and which lessens the hazard	[ ]	[ ]
6. On biological hazards		
a) Pathogens are biological agents that are not capable of causing diseases	[ ]	[ ]
b) They include bacteria, viruses and fungi as well as larger organisms like parasites and plants	[ ]	[ ]
c) Biological hazard can be controlled at source through elimination, isolation or containment of the hazard	[ ]	[ ]
d) Biological agents can enter the body by ingestion and inhalation and not by absorption	[ ]	[ ]
e) Dermatophytosis is a fungus that mostly affects swimming pool attendants and dish washers.	[ ]	[ ]

	TRUE	FALSE
10. On lead poisoning		
a) Encouraging children to eat plenty of foods that contain iron and calcium will reduce the risk of lead exposure	[ ]	[ ]
b) The most serious effects of lead exposure in children are vomiting and weight loss	[ ]	[ ]
c) Kidney damage and peripheral nerve disorders are two effects of lead exposure in adults	[ ]	[ ]
d) Lead exposure can affect a persons ability to have children	[ ]	[ ]
e) As long as children don't eat paint chips, they aren't in any danger from lead based paints	[ ]	[ ]

PART B: ESSAY QUESTIONS

- Answer all the questions

1. a) With examples differentiate between occupational diseases and occupational related diseases. (10 marks)

   b) Write an essay on organophosphate poisoning clearly highlighting its causes, signs and symptoms, pathophysiology, diagnosis and treatment. (20 marks)

2. a) Define Byssinosis (5 marks)

   b) Write a short essay on asbestosis indicating its symptoms, group at risk, prevention and management. (15 marks)

- END -

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KENYA MEDICAL TRAINING COLLEGE

PORT REITZ CAMPUS

DEPARTMENT OF CLINICAL MEDICINE

EXAM:	END OF SEMESTER SIX (3RD YEAR)
CLASS:	SEPTEMBER 2015
PAPER:	PAEDIATRICS
DATE:	11TH MAY 2016
TIME:	9.00 –12.00 NOON
DAY:	WEDNESDAY
PART A:	150 MARKS
PAR B:	20 MARKS
TOTAL MARKS:	170 MARKS
TOTAL NO. OF PAGES:	10

INSTRUCTIONS

1. USE REGISTRATION NUMBERS AND NOT NAMES IN THIS EXAMINATION.
2. ANSWER PART A (TRUE/FALSE) ON THE ANSWER SHEET PROVIDED BY MARKING "X" WHERE APPLICABLE.
3. ANSWER PART B (MCQ) ON THE EXAMINATION ANSWER SHEET PROVIDED.
4. MOBILE PHONES, TEXT BOOKS OR ANY LITERATURE ARE FORBIDDEN IN THE EXAMINATION ROOM.

PART A: MULTIPLE TRUE-FALSE QUESTIONS

Q1) The following drug is recommended as the first line drug in management of severe malaria.

a) DHA-piperaquine
b) Coartem
c) Chloroquine injection
d) Artesunate injection
e) Quinine injection

Q2) The following are live attenuated vaccines:

a) BCG
b) Rubella
c) Diphtheria
d) Tetanus
e) Opv

Q3) A bout the cold chain:

a) Diluents and emergency drugs can be stored on the lower shelves.
b) Vaccines should be frozen
c) All freezer sensitive vaccines should be stored on the middle shelves
d) During power failure of 4 hours or less ,the refrigerator door should be kept closed
e) If power failure occurs more than 4 hours ,refrigerator door should be kept open to preserve vaccine potency.

Q4) An 18 month old baby boy has swallowed a whole bottle of Feroglobin syrup. 6 hours later the child is drowsy and twitches are noted:

a) This is cerebral malaria
b)Deferoxamine is indicated
c)Iron poisoning is highly suspected
d)Gastric ravage is contraindicated
e)Salicylate poisoning is suspected

Q5) Paralytic ilea's may be caused by each of the following except:

a) U.T.I
b) Hypokalaemia
c) Peritonitis
d) Electrolyte imbalance
e) Severe gastroenteritis.

Q6) The predominant hemoglobin present at birth in normal infant is:

a) Hbs
b) Hbc
c) HbF
d) HbA
e) Hbl

Q7) The following are True or False of Iron deficiency anemia :

a) Low MCV
b) Nutritional deficiency
c) Chronic hemorrhage.
d) Koilonychia
e) Malaria

Q8)In sickle cell disease:

a) The earliest symptoms are usually swelling of both hands and feet
b) Neonates usually present with hemolytic jaundice
c) Chloroquine is a administered weekly as a prophylaxis
d) Iron and Folic acid are administered as a routine to prevent anemia
e) Onset of symptoms and signs depends on the proportion of sickle hemoglobin in the blood

Q9) Aspirin poisoning:

a) Can cause tachycardia
b) Hemorrhagic manifestations do occur
c) Injection with atropine is mandatory
d) Haematemesis may be present
e) Pulmonary oedema is common

Q10) In kerosene poisoning:

a) Emesis should be induced immediately
b) Can be fatal
c) Is often complicated by respiratory distress syndrome
d) A activated charcoal should be given immediately
e) Ingestion can cause encephalopathy.

Q11)The following laboratory features are consistent with iron deficiency anemia

a) Target cells
b) Hypochromia
c) Anisocytosis
d) Microcytosis
e) Poikilocytosis

Q12)The following are common causes of Anemia in a neonate

a) Severe malaria
b) Prematurity
c) Fetal maternal transfusion
d) Intestinal worms
e) Ante-partum hemorrhage

Q13) When making a diagnosis of non severe pneumonia

a) The respiratory rate should be high
b) History of cough or difficulty in breathing is present
c) Tachycardia is important
d) Low chest wall in drawing is present
e) Crepitations on auscultation is important.

Q14) About acute diphtheria infection:

a) Should be treated with oral amoxicillin
b) Is caused by a virus
c) Is the major cause of a cute croup
d) Is associated with formation of a membrane in the throat
e) Invariably presents with stridor

Q15)Stridor:

a) Is assign of upper airway obstruction
b) Is assign of laryngeal infection only
c) Is an inspiratory sound
d) When it occurs after the first few days of life ,it could be due to tracheomalacia
e) Occurs during inspiration

Q16)The following are causes of intra uterine fetal infections:

a) Toxoplasmosis
b) HIV/AIDS
c) Rubella
d) Candidiasis
e) Syphilis

Q17) In the new born baby;

a) Prophylactic antibiotic eye ointments are to be administered routinely
b) The normal baby requires minimal interference and should be placed on the mother's arms immediately after birth
c) Neurological examination is important for the assessment of maturity
d) The hymenal tag in the female neonates should be referred to the surgeon for urgent surgical intervention
e) A systolic murmur can be commonly heard in the normal newborns.

Q18)The following condition may present with Rice water diarrhea:

a) Food poisoning
b) Amoebic dysentery
c) Chronic diarrhea
d) Cholera
e) Drug induced diarrhea

Q19) About caput succedaneum

a) Usually needs antibiotics
b) Usually disappears in a few days without problems
c) Vacuum extraction is the major cause
d) Jaundice is a feature
e) Surgical intervention is required urgently in its treatment

Q20)The following are true about a newborn baby:

a) A palpable liver up to 2cm is abnormal
b) Presence of vernix caseosa needs urgent treatment
c) P V bleeding in newborn females needs detailed investigations even in absence of disease
d) The weight gain starts immediately after birth in the absence of disease
e) In the first 3 days the lymphocytes are high

Q21)Prematurity leads to anaemia due to :

a) Short RBC lifespan
b) Vitamin k deficiency
c) Maternal anemia
d) Excessive bleeding from cord at birth
e) Rapid increase in body mass vis a vis bone marrow production

Q22)The following are differentials of a 8 month old infant with a wheeze:

a) Aspiration of secretions
b) Foreign body
c) Bronchitis
d) Asthma
e) Chronic Bronchitis

Q23) Daniela a 4 year old girl presents with headache, pain and discharging right ear, Fever and ongoing Rhinitis.

a) She is suffering from sinusitis
b) Chloramphenicol and Gentamicin ear drops should be prescribed
c) Oral Amoxicillin would be adequate treatment
d) Use of expectorants are beneficial in relieving the symptoms
e) Ear syringing is beneficial to remove the discharge

Q24) In relation to vaccines:

a) Pentavalent vaccine is commenced routinely at 2 months of age
b) MMR is included in KEPI schedule
c) Permanent protection follows immunization against Tuberculosis
d) Polio vaccine should not be given to children who have suffered poliomyelitis because it confers permanent immunity
e) Measles vaccine can be given if the child has an active measles disease

Q25)About cholera:

a) Heat labile enterotoxin causes watery diarrhea
b) Is a viral infection and can be controlled by vaccine administration
c) Transmission is by oral fecal route
d) Isolation is not necessary
e) Inaba ,ogawa and Hikojima are the known serotypes

Q26)About measles:

a) Koplik spots are pathognomonic
b) Children under one year are the most susceptible group
c) Blindness is due to the measles infection alone
d) Can cause diarrhea/dehydration as a complication
e) Is not a among the notifiable diseases

Q27) A bout poliomyelitis:

a) Sensation is intact
b) Phantom hernis is a feature
c) Incubation period is 7-14 days
d) Transmission is through oral fecal route
e) 90 to 95 % of individuals develop in apparent infection.

Q28)The sickle cell precipitants include:

a) Low oxygen levels
b) Acidosis
c) Infections
d) Malnutrition
e) Dehydration

Q29) When managing a chocking infant:

a) Laying the infant in head down position on your arm or thigh is recommended
b) Giving five blows using the heel of the hand to the middle of the infants back is recommended
c) Checking and removing any obstruction in the mouth is essential.
d) Suctioning deep down the pharynx to remove the foreign
e) Body is highly recommended

Q30 Signs of some dehydration include:

a) Irritability
b) Lethargy
c) Thirst and drink eagerly
d) Skin pinch goes back very slowly
e) Sunken eyes

SECTION B:MULTIPLE CHOICE QUESTIONS

Q1) The earliest sign of malnutrition

a) Underweight
b) Oedema of both limbs
c) Hair changes
d) Skin changes

Q2)The following are notifiable diseases in Kenya except:

a) Measles
b) Cholera
c) Neonatal Tetanus
d) Malaria

Q3)The mother of an infant with Ricketts is most likely to be concerned with:

a) Bowed legs
b) Delayed developmental milestones
c) Swollen ankles
d) Large head

Q4) About cholera:

a) Incubation period is 12-48hours
b) Diarrhea is watery, effortless, profuse and associated with pain.
c) Death is solely caused by the vibrio cholera
d) Vibrio cholera is a gram positive flagellated bacillus.

Q5)The following forms the two main sources of oxygen:

a) Nasal prongs and mask with reservoir
b) Concentrators and cylinders
c) Nasopharyngeal and nasal catheters
d) Pulse oximetry and concentrators

Q6) The recommended second line treatment for uncomplicated malaria in Kenya is:

a) Dihydroartemesinin-piperaquine
b) Artemether
c) Artesunate orally
d) Quinine infusion

Q7) A bout quinine administration

a) Quinine is contraindicated in severe anaemia
b) in renal insufficiency the dose remains unchanged
c) Hyperglycemia is a potential side effect
d) Can be given as a bolus infusion

Q8) In Rheumatic heart disease, which of the following valves rarely affected:

a) Tricuspid valve
b) Aortic valve
c) Pulmonary valve
d) Mitral valve

Q9) About A cute Rheumatic fever:

a) Migratory polyarthritis is a feature in minor criteria
b) St. Vitus dance is not a feature
c) ESR is elevated
d) The skin is not affected

Q10) Which of the following congenital condition causes 8th nerve deafness as a complication:

a) Congenital Rubella
b) Toxoplasmosis
c) Cytomegalovirus
d) Syphilis

Q11) Majority of diarrhoeal episodes are caused by:

a)Viruses
b)Bacteria
c)Parasites
d)Trematodes

Q 12) A bout diarrhoea:

a) Mainly affects children less than 2 years .
b) Mainly affects children less than 1year.
c) Kaolin is recommended in a cute diarrhoea
d) In malnutrition, Intravenous fluids are recommended in severe dehydration management.

Q13. The following are priority signs except:
a). Severe pain
b). Severe pallor
c). Central cyanosis
d). Oedema of both feet

Q14. Jaundice is very common during the neonatal period. One of the most important cause of rapidly increasing jaundice in the first 24 hours of life is:
a) Neonatal sepsis
b) Malaria
c) Hypothyroidism
d) Rhesus incompatibility

Q15. Which of the following is not a cardinal feature of congestive heart failure?
a) Cardiomegally
b) Tachypnoea
c) Hepatomegally
d) Oedema of the facial and lower limbs

Q16. Prolonged use of corticosteroids in children leads to the following except:
a) Decreased immune response
b) Skin atrophy
c) Hypersensitivity reaction
d) Growth retardation

Q17. Which of the following is a normal developmental milestone?:
a) First words at 12 months
b) Grip by 7 months
c) Sitting with support at 9 months
d) Smile at 7 months

Q18. Which of the following vaccine is administered during neonatal period:
a) Measles vaccine
b) Pertussis vacine
c) BCG vaccine
d) Diphtheria vaccine

Q19. Lung collapse due to distal air absorption is called:
a) Bronchiectasis
b) Laryngo spasms
c) Pneumatocele
d) Atelectasis

Q20. Delayed development milestone in a child could be due to the following except:

a) Child feeding or breast milk upto 4 months
b) Child going to Nursery School
c) Child neglect
d) Recurrent or chronic infection

PART II - ESSAY QUESTIONS

Q1. a) Define pneumonia and give top three bacterial causes.

b) Explain the clinical classification of pneumonia in children under 5 years of age.

c) Write a treatment sheet for a two (2) year old child of appropriate weight for age who has severe pneumonia.

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PORT REITZ MEDICAL TRAINING COLLEGE

DEPARTMENT OF CLINICAL MEDICINE

2ND YEAR 6TH SEMESTER (MOCK) EXAMINATION – SEPT. 2010 CLASS

DATE: 7-5-2013 PAEDIATRICS TIME: 3 HRS

PART IIA – TRUE/FALSE QUESTIONS

		TRUE	FALSE
1.	A one week old male infant presents with a breast enlargement which when pressed yields whitish fluid. He is otherwise normal. Which of these statements are true?:		
	(a) The fluid is likely to be pus	[ ]	[ ]
	(b) Should be squeezed daily until dry	[ ]	[ ]
	(c) Baby should be started on antibiotics after a swab	[ ]	[ ]
	(d) Could be witches milk and should be left undisturbed	[ ]	[ ]
	(e) Referral to the Paediatric Surgeon is essential	[ ]	[ ]
2.	The earliest sign of malnutrition is:		
	(a) Hair changes	[ ]	[ ]
	(b) Under weight	[ ]	[ ]
	(c) Diarrhoea and vomiting	[ ]	[ ]
	(d) Oedema	[ ]	[ ]
	(e) Skin changes	[ ]	[ ]
3.	Causes of large for gestational age baby:		
	(a) Maternal diabetes	[ ]	[ ]
	(b) Smoking	[ ]	[ ]
	(c) Over eating in the mother	[ ]	[ ]
	(d) Alcohol	[ ]	[ ]
	(e) All of the above	[ ]	[ ]
4.	Delayed milestones in a child could be due to:		
	(a) Cerebral palsy	[ ]	[ ]
	(b) Child neglect	[ ]	[ ]
	(c) Exclusive breast feeding for 4 months	[ ]	[ ]
	(d) Recurrent or chronic infection	[ ]	[ ]
	(e) Eating well	[ ]	[ ]

		TRUE	FALSE
5.	Paralytic ileus may be caused by each of the following except:		
	(a) UTI	[ ]	[ ]
	(b) Severe gastroenteritis	[ ]	[ ]
	(c) Peritonitis	[ ]	[ ]
	(d) Electrolyte imbalance	[ ]	[ ]
	(e) Hypokalaemia	[ ]	[ ]
6.	The following are true about a new born baby:		
	(a) In the 1st 3 days lymphocytes are predominantly high	[ ]	[ ]
	(b) P.V. bleeding in female newborns requires detailed investigations even in absence of any sign disease	[ ]	[ ]
	(c) A palpable liver of upto 2 cm is normal	[ ]	[ ]
	(d) The weight gain starts immediately after birth in the absence of any disease	[ ]	[ ]
	(e) Presence of vermix caseo sa needs immediate treatment	[ ]	[ ]
7.	Causes of abdominal distension in a child with kwashiorkor include:		
	(a) Organomegally	[ ]	[ ]
	(b) Intestinal worms	[ ]	[ ]
	(c) Protein breakdown with reduced muscle mass	[ ]	[ ]
	(d) Shifting of fluid into extracellular space	[ ]	[ ]
	(e) Fatty infiltration of the liver	[ ]	[ ]
8.	When taking history from a child with coughs:		
	(a) Weight loss or failure to gain weight is an important aspect	[ ]	[ ]
	(b) Colour and smell of the sputum is an important clue in a child aged 4½ yrs	[ ]	[ ]
	(c) A day early morning cough should prompt the impression of presence of wheezing	[ ]	[ ]
	(d) Left sided chest pain is indicative of a angina pectories	[ ]	[ ]
	(e) Difficult breathing must be present in an impression of pneumonia is to be considered	[ ]	[ ]
9.	The following is true in asphyxia neonatorum:		
	(a) May result in haematuria	[ ]	[ ]
	(b) APGAR score is a reliable indicator of the problem	[ ]	[ ]
	(c) There is usually metabolic acidosis in established cases	[ ]	[ ]
	(d) It is a safe cause of mortality	[ ]	[ ]
	(e) Most infants (newborns) requires sodium bicarbonate within the first minute	[ ]	[ ]

		TRUE	FALSE
10.	The following are true about early neonatal sepsis:		
	(a) The commonest organism is E. coli	[ ]	[ ]
	(b) A week of antibiotic treatment is usually effective	[ ]	[ ]
	(c) Initial presentation is refusal to feed	[ ]	[ ]
	(d) It is best to wait for specific screening results prior to commencing treatment	[ ]	[ ]
	(e) Penicillin and Gentamycin usually constitute first line treatment	[ ]	[ ]
11.	The following are normal:		
	(a) Head circumference of 39 cm at birth	[ ]	[ ]
	(b) Respiratory rate of 49/minute in a 2 years old patient	[ ]	[ ]
	(c) Heart rate of 150 b/min at birth	[ ]	[ ]
	(d) Ability to walk from 30 months of age	[ ]	[ ]
	(e) Weight of 8.5 kg at 1 year	[ ]	[ ]
12.	The following are causes of intrauterine fetal infections:		
	(a) Syphilis	[ ]	[ ]
	(b) Candidiasis	[ ]	[ ]
	(c) Rubella	[ ]	[ ]
	(d) HIV and AIDS	[ ]	[ ]
	(e) Toxoplasmosis	[ ]	[ ]
13.	In the new born baby:		
	(a) Prophylactic antibiotic eye ointments are to be administered routinely	[ ]	[ ]
	(b) The normal baby requires minimal interference and should be placed on the mother's arms immediately after birth	[ ]	[ ]
	(c) Asytolic murmur can commonly be heard in the normal newborn	[ ]	[ ]
	(d) The hymonal tag in the female newborns should be Left alone	[ ]	[ ]
	(e) Neurological examination is necessary for the assessment of maturity	[ ]	[ ]
14.	The following drugs are useful for the treatment of malaria today :		
	(a) Chloroquine injection	[ ]	[ ]
	(b) I.V. Quinine	[ ]	[ ]
	(c) I.M. Quinine	[ ]	[ ]
	(d) Co-artem	[ ]	[ ]
	(e) Chloroquine tablets	[ ]	[ ]

		TRUE	FALSE
15.	TRUE OR FALSE:		
	(a) I.V. Quinine loading dose should be given 20 mg/kg bolus	[ ]	[ ]
	(b) DHA-Piperaquine is the second line drug in the management of complicated malaria	[ ]	[ ]
	(c) The loading dose for Quinine infusion is 15 mg/kg/bist	[ ]	[ ]
	(d) Loading dose of Quinine should be omitted if Quinine has already been administered in 24 hrs	[ ]	[ ]
	(e) Intravenous Quinine should not be administered in first trimester to a pregnant mothers	[ ]	[ ]
16.	Oral thrush in the newborn:		
	(a) Should be treated with nysatin drops	[ ]	[ ]
	(b) Always indicate immunodeficiency	[ ]	[ ]
	(c) May cause diarrhoea	[ ]	[ ]
	(d) Responds well to Metronidazole	[ ]	[ ]
	(e) Commonly presents with inability to feed	[ ]	[ ]
17.	Measles infection:		
	(a) Also known as Rubella	[ ]	[ ]
	(b) Also known as Rubeola	[ ]	[ ]
	(c) Has a 21 day incubation period	[ ]	[ ]
	(d) Is a non communicable disease	[ ]	[ ]
	(e) Diagnosis requires laboratory confirmation	[ ]	[ ]
18.	A six month old boy presents with a 3 days history of diarrhoea and vomiting. On examination he is irritable, has acidotic breathing but is able to drink:		
	(a) Has severe dehydration and therefore ORS is not useful	[ ]	[ ]
	(b) Rota virus is the most likely causative organism	[ ]	[ ]
	(c) Immediate antibiotic cover is indicated for possible bacterial aetiology	[ ]	[ ]
	(d) Breastfeeding is best withheld temporarily until diarrheoa subsides	[ ]	[ ]
	(e) Loperamide would be useful to stop the diarrhoea	[ ]	[ ]

		TRUE	FALSE
19.	In mainutrition:		
	(a) An obese child body mass index is more than 95th percentile	[ ]	[ ]
	(b) Under weight children have body mass index less than 5th percentile	[ ]	[ ]
	(c) Child with kwashiorkor have hypoalbuminaemia	[ ]	[ ]
	(d) A child with marasmus has body weight below 60% of the expected weight	[ ]	[ ]
	(e) Angular cheilosis is a feature in marasmus	[ ]	[ ]
20.	A 3 year old baby presents with diarrhea and vomiting, On examination, the children may find the following:		
	(a) Irritability	[ ]	[ ]
	(b) Cold extremities	[ ]	[ ]
	(c) Sunken eyes	[ ]	[ ]
	(d) Altered level of consciousness	[ ]	[ ]
	(e) Unaltered skin Tigour	[ ]	[ ]
21.	The following are viral infectious diseases:		
	(a) Measles	[ ]	[ ]
	(b) Whooping cough	[ ]	[ ]
	(c) Diptheria	[ ]	[ ]
	(d) Mumps	[ ]	[ ]
	(e) Poliomyelitis	[ ]	[ ]
22.	The following drugs are recommended for the treatment of severe malaria:		
	(a) Injection Quinine Sulphate	[ ]	[ ]
	(b) Artemeter/lumefatrine tablets	[ ]	[ ]
	(c) Duocotexcin tablets	[ ]	[ ]
	(d) Injection Artemesinin	[ ]	[ ]
	(e) Injection Artemether	[ ]	[ ]
23.	The following are causes of failure to thrive:		
	(a) Emotional deprivation	[ ]	[ ]
	(b) Parental nutritional beliefs	[ ]	[ ]
	(c) Maternal literacy level	[ ]	[ ]
	(d) Genital urinary tract infections	[ ]	[ ]
	(e) Sickle cell disease	[ ]	[ ]

24. The following are notifiable diseases in Kenya:
(a) Neonatal tetanus [ ] [ ]
(b) Measles [ ] [ ]
(c) Cholera [ ] [ ]
(d) Poliomyelitis [ ] [ ]
(e) Swaine flu [ ] [ ]

25. A 2 year old girl weighing 12 kg presents with diarrhoea and vomiting. On examination he is irritable, thirsty and skin pinch goes back slowly. Management of the child includes:
(a) ORS 50 – 100 mls after each bout of diarrhoea [ ] [ ]
(b) ORS 900 mls in 4 hours [ ] [ ]
(c) Zinc sulphate tablets 20 mg OD x 2/52 [ ] [ ]
(d) Vitamin A_1 200,000 iu ODx1/52 [ ] [ ]
(e) I.V. Hartmans solution 360 mls in 1st 30 minutes then 480 mls in the next 2½ hours [ ] [ ]

26. The following is the first line drug used in the management of severe malaria today:
(a) I.V. Quinine infusion [ ] [ ]
(b) I.M. Quinine [ ] [ ]
(c) I.M. or I.V. Artemether [ ] [ ]
(d) I.V. Artemether [ ] [ ]
(e) Artemether/Lumefantrine [ ] [ ]

27. A ten year old girl has been brought to Paediatric Filter Clinic with complaints of headache, vomiting, pain in the neck and hotness of the body. Which of the following investigations will you perform in her:
(a) Blood slide for malaria parasites [ ] [ ]
(b) Blood for urea and electrolytes [ ] [ ]
(c) Lumbar puncture for biochemistry [ ] [ ]
(d) Plain abdominal x-ray. [ ] [ ]
(e) Blood for glucose levels [ ] [ ]

		TRUE	FALSE
28.	Mark, a 3 year old baby presents to you with complaints of: diarrhoea and vomiting. On examination, he is unresponsive, with cold extremities, weak radial pulse and has a capillary refill of 4 seconds. He weighs 14 kg. The following would form your immediate treatments for Mark:		
	(a) I.V. 10% dextrose 70 mls bolus stat	[ ]	[ ]
	(b) Stabilise ABC	[ ]	[ ]
	(c) I.V. Hartmans solution 250 mls	[ ]	[ ]
	(d) I.V. Hartmans solution 210 mls bolus 1 hour then reassess	[ ]	[ ]
	(e) F 75 228 mls 3 hourly via NG tube	[ ]	[ ]
29.	The mother of an infant with rickets is most likely to be concerned with:		
	(a) Swollen ankles	[ ]	[ ]
	(b) Large head	[ ]	[ ]
	(c) Bowed legs	[ ]	[ ]
	(d) Swollen wrists	[ ]	[ ]
	(e) Delayed development	[ ]	[ ]
30.	When initiating the feeding of a child with acute severe wasting with oedema, the amount of F 75 should be:		
	(a) 90 mls/kg/day	[ ]	[ ]
	(b) 100 mls/kg/day	[ ]	[ ]
	(c) 110 mls/kg/day	[ ]	[ ]
	(d) 120 mls/kg/day	[ ]	[ ]
	(e) 130 mls/kg/day	[ ]	[ ]

PART IIB – ESSAY QUESTIONS

Q1. Purity is a two year old girl. Yesterday she developed a severe bout of diarrhea and vomiting. This continued throughout the night. She vomited everything. Her mother took Purity to a nearby health centre. The Clinician took history as given above. On examination the Clinician made the following observations. Avpu, semicomatose sunken eyeballs, dry mucus membranes of mouth and tongue, extremities very cold, capillary refill > 3 sec, skin turgor > 3 sec pulse < weak and throach weight 11 kg.

(a) Give an appropriate working diagnosis. (2 marks)

(b) Write a comprehensive treatment sheet for baby Purity. (8 marks)

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POPE REITZ MEDICAL TRAINING COLLEGE

DEPARTMENT OF CLINICAL MEDICINE

3RD YEAR 6TH SEMESTER MOCK EXAMINATION – SEPT. 2010 CLASS

DATE: 7-5-2013 PAEDIATRICS TIME: 3 HRS

PART IA – TRUE/FALSE QUESTIONS

		TRUE	FALSE
1.	In transfusion:		
	(a) The baby must be monitored every five minutes for the first 15 minutes	[ ]	[ ]
	(b) The initial general appearance of the baby is not usually recorded	[ ]	[ ]
	(c) Fluid balance is of little importance	[ ]	[ ]
	(d) Time of start and time of stop is absolutely important	[ ]	[ ]
	(e) Blood should be left hanging for more than 4 hrs	[ ]	[ ]
2.	The following are true:		
	(a) In emergency uncrossmatched blood type O Rh negative may be used and subsequent grouping and x-matching is not necessary	[ ]	[ ]
	(b) Blood is transfused at 20 mls/kg within 4 hrs unless in emergency situation	[ ]	[ ]
	(c) The only deciding factor for transfusion is the Hb or Haematocrit	[ ]	[ ]
	(d) In small healthy babies (born < 2.5 kg and gestation of < 37/52) are to be transfused if Hb is < 8g/dl	[ ]	[ ]
	(e) The severely malnourished child similar to the well Nourished are transfused at the same rate when severely dehydrated	[ ]	[ ]
3.	Of rheumatic diseases:		
	(a) Autoimmunity is a characteristic	[ ]	[ ]
	(b) Inflammation acute or chronic are characteristics	[ ]	[ ]
	(c) Vasculitisis a feature	[ ]	[ ]
	(d) Failure to thrive may be a feature	[ ]	[ ]
	(e) Fever is a usual feature	[ ]	[ ]

		TRUE	FALSE
4.	In chromosomal abnormalities:		
	(a) Turner's syndrome is more severe and causes more fetal loss than Down's Syndrome	[ ]	[ ]
	(b) Turner's syndrome is the commonest chromosomal abnormality though most are aborted	[ ]	[ ]
	(c) Of live children born with chromosomal abnormality Down's syndrome is the most common	[ ]	[ ]
	(d) Feature are usually failure to thrive, small babies, developmental delay and congenital malformations	[ ]	[ ]
	(e) Down's syndrome is also called Trisomy – 21	[ ]	[ ]
5.	Of jaundice:		
	(a) Bilirubin is > 85 umol/l	[ ]	[ ]
	(b) G6PD deficiency may suspected if it occurs on day of birth in a male neonate	[ ]	[ ]
	(c) Only results from breakdown of haemoglobin	[ ]	[ ]
	(d) In the neonate the major risk factors are small baby, heamolysis and sepsis	[ ]	[ ]
	(e) May cause bilirubin encephalopathy due to high serum levels	[ ]	[ ]
6.	The following are true:		
	(a) At delivery immediately there is need to slap the baby	[ ]	[ ]
	(b) At delivery its necessary to suction the baby at the stage of restitution	[ ]	[ ]
	(c) Suction right down the throat due to meconium aspiration at delivery may cause apinoea and bradycardia	[ ]	[ ]
	(d) In newborn resuscitation, the ventilations are given at 40 – 60 breaths/min	[ ]	[ ]
	(e) In resuscitation of the newborn the ratio of chest compressions to ventilation (breaths) are 90:30 every minute (3:1 every 2 seconds)	[ ]	[ ]
7.	The following are NOT emergency signs in the assessment of the newborn:		
	(a) Breathing spontaneously	[ ]	[ ]
	(b) Gasping	[ ]	[ ]
	(c) Respiratory rate < 20/min	[ ]	[ ]
	(d) Bleeding	[ ]	[ ]
	(e) Shock	[ ]	[ ]

		TRUE	FALSE
8.	In the newborn:		
	(a) When resuscitating the head is placed in the "sniffing" position	[ ]	[ ]
	(b) The emergency signs are treated first then the convulsions	[ ]	[ ]
	(c) Convulsions are preferably treated with P.R diazepam at 0.5 mg/kg than I.V diazepam at 0.3 mg/kg	[ ]	[ ]
	(d) Commonest cause of convulsion is metabolic disorders	[ ]	[ ]
	(e) Convulsions must be distinguished from spasms	[ ]	[ ]
9.	Necrotizing enterocolitis:		
	(a) Especially affect babies after commencement of enteral feeding	[ ]	[ ]
	(b) Is less common in small (low birth weight) babies on artificial formulae	[ ]	[ ]
	(c) Never affect breastfed babies	[ ]	[ ]
	(d) May present with apnoea, drowsiness and fever	[ ]	[ ]
	(e) Blood in stool is not a feature	[ ]	[ ]
10.	Neonatal septiceamia:		
	(a) Is viremia of the newborn within 90 days of birth	[ ]	[ ]
	(b) Low birth weight is a risk factor	[ ]	[ ]
	(c) Meningitis is a cause	[ ]	[ ]
	(d) Early onset occurs after seven days of birth	[ ]	[ ]
	(e) The male baby is more at risk	[ ]	[ ]
11.	The following are the composition of pulmonary intesrtitium:		
	(a) Walls of alveoli	[ ]	[ ]
	(b) Alveolar sacs	[ ]	[ ]
	(c) Bronchi	[ ]	[ ]
	(d) Alveolar ducts	[ ]	[ ]
	(e) Bronchioles	[ ]	[ ]
12.	The following are causes of difficulty in breast feeding:		
	(a) FTT	[ ]	[ ]
	(b) Correct positioning and attachment	[ ]	[ ]
	(c) GIT malformation or obstruction	[ ]	[ ]
	(d) Twin	[ ]	[ ]
	(e) Sepsis	[ ]	[ ]

		TRUE	FALSE
13.	Respiratory failure:		
	(a) May be hypercarbic – $PaCO_2 < 50$ mmHg	[ ]	[ ]
	(b) May be hypoxeamic $PaO_2 > 60$mmHg	[ ]	[ ]
	(c) Gasping may be a feature	[ ]	[ ]
	(d) Is not distinctly different from respiratory distress syndrome	[ ]	[ ]
	(e) May be due to :"dead space" ventillation	[ ]	[ ]
14.	The following are true:		
	(a) Cyanosis is only elicited at Hb > 5g/dl	[ ]	[ ]
	(b) Hypercarbic respiratory failure results into increased depth of breathing more than hypoxeamic respiratory failure	[ ]	[ ]
	(c) Respiratory distress syndrome is typically seen in normal birth weight babies	[ ]	[ ]
	(d) Respiratory distress syndrome starts within hours of birth	[ ]	[ ]
	(e) Respiratory distress syndrome never results into apnoea	[ ]	[ ]
15.	Of mumps:		
	(a) Only spread due to contact with infected person through respiratory sections	[ ]	[ ]
	(b) Spread may occur from in infected person 6 days before onset of symptoms	[ ]	[ ]
	(c) Spread after onset of symptoms	[ ]	[ ]
	(d) Typically incubation period is 16 – 18 days (or 14 – 25 days)	[ ]	[ ]
	(e) Aerosol spread may cause disease	[ ]	[ ]
16.	The following are complications of mumps:		
	(a) Meningitis	[ ]	[ ]
	(b) Spontaneous abortion	[ ]	[ ]
	(c) Encephalitis	[ ]	[ ]
	(d) Epididymorchitis	[ ]	[ ]
	(e) Pancreatitis	[ ]	[ ]
17.	The following are not known cause contributors to failure to thrive:		
	(a) Environmental	[ ]	[ ]
	(b) Medical	[ ]	[ ]
	(c) Developmental	[ ]	[ ]
	(d) Psychological	[ ]	[ ]
	(e) Nutritional	[ ]	[ ]

		TRUE	FALSE
18.	The following statements are true:		
	(a) Phenytoin is given at 15 mls in normal Saline	[ ]	[ ]
	(b) Only normal Saline is used with Phenytoin since other fluids cause it to crystallize	[ ]	[ ]
	(c) Diazepam is used in neonates initially then changed to Phenobarb after the second dose	[ ]	[ ]
	(d) Diazepam given with Phenobarb may cause respiratory failure and also circulatory collapse	[ ]	[ ]
	(e) Loading of Phenobarb is 10 mg/kg and maintenance dose is 20 mg/kg	[ ]	[ ]
19.	The following are investigations done in cardiovascular disease:		
	(a) Chest x-ray	[ ]	[ ]
	(b) Pulse oximetry	[ ]	[ ]
	(c) E.C.G.	[ ]	[ ]
	(d) E..E.G.	[ ]	[ ]
	(e) Echocardiogram	[ ]	[ ]
20.	The following are true:		
	(a) Measles affect only human beings	[ ]	[ ]
	(b) Mumps affect animals other than human beings	[ ]	[ ]
	(c) Poliomyelitis only affect human beings	[ ]	[ ]
	(d) Poliomyelitis only affects the nervous system and musculoskeletal system	[ ]	[ ]
	(e) In "Black Measles" the maculopapular rash is haemorhagic	[ ]	[ ]
21.	Death from measles occur from:		
	(a) Encephalitis	[ ]	[ ]
	(b) B'pneumonia	[ ]	[ ]
	(c) Subacute sclerosing panencephalitis	[ ]	[ ]
	(d) Myocarditis	[ ]	[ ]
	(e) Gastroenteritis	[ ]	[ ]
22.	Measles vaccine may still be given in:		
	(a) Pregnancy	[ ]	[ ]
	(b) Severe HIV infection	[ ]	[ ]
	(c) Leukaemia	[ ]	[ ]
	(d) Corticosteroid therapy	[ ]	[ ]
	(e) Chemotherapy	[ ]	[ ]

		TRUE	FALSE
23.	The following are true of infective endocarditis:		
	(a) Common causae is strep viridans for all ages	[ ]	[ ]
	(b) It's an infection of the endothelial lining of the heart including the valves	[ ]	[ ]
	(c) It's primarily a complication of congenital heart disease	[ ]	[ ]
	(d) It's primarily caused by rheumatic heart disease	[ ]	[ ]
	(e) In adults male/female ratio is higher in males	[ ]	[ ]
24.	The following minor criteria in Ducket- Jones criteria for diagnosis of rheumatic fever:		
	(a) Polyarthritis	[ ]	[ ]
	(b) Fever	[ ]	[ ]
	(c) Arthralgia	[ ]	[ ]
	(d) Chorea	[ ]	[ ]
	(e) Elevated ESR	[ ]	[ ]
25.	The following are cyanotic heart diseases:		
	(a) VSD	[ ]	[ ]
	(b) ASD	[ ]	[ ]
	(c) TOF	[ ]	[ ]
	(d) Truncus anteriosus	[ ]	[ ]
	(e) PDA	[ ]	[ ]
26.	The following are recognized mechanisms of diarrhoea:		
	(a) Osmotic	[ ]	[ ]
	(b) Secretory	[ ]	[ ]
	(c) Reduced motility	[ ]	[ ]
	(d) Mucosal inflammation	[ ]	[ ]
	(e) Mucosal adherence	[ ]	[ ]
27.	The following are true:		
	(a) Persistent diarrhea is termed "severe persistent diarrhoea" only if there is some or severe dehydration	[ ]	[ ]
	(b) Routine administration of antibiotics are recommended with persistent diarrhoea.	[ ]	[ ]
	(c) Supplementary micronutrients must be given to all patients with persistent diarrhoea	[ ]	[ ]
	(d) Temperature and weight must be done in the wards to monitor a patient with persistent diarrhoea	[ ]	[ ]
	(e) Food taken and the number/episodes of diarrhea are not necessary to monitor patient with persistent diarrhoea who is in the wards	[ ]	[ ]

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KMTC/QP-08/TIS

KENYA MEDICAL TRAINING COLLEGE

POPT REITZ CAMPUS

DEPARTMENT OF CLINICAL MEDICINE

END OF SEMESTER SIX IN 3RD YEAR
FOR THE 2013 CLASS

PAPER:	**PAEDIATRICS**
DATE:	**11TH MAY 2016**
TIME:	**9.00 A.M. – 12.00 NOON**
DAY:	**WEDNESDAY**
PART I:	**100 MARKS**
PART II:	**50 MARKS**
TOTAL MARKS:	**150 MARKS**
TOTAL NO. OF PAGES:	**7**

INSTRUCTIONS

1. USE REGISTRATION NUMBERS AND NOT NAMES IN THIS EXAMINATION.
2. ANSWER PART I (MULTIPLE T/F) ON THIS QUESTION PAPER BY WRITING "X" WHERE APPLICABLE
3. ANSWER PART II ON THE FULLSCAPS PROVIDED
4. MOBILE PHONES, TEXT BOOKS OR ANY LITERATURE ARE FORBIDDEN IN THE EXAMINATION ROOM.

PART I – MULTIPLE TRUE/FALSE QUESTIONS (100 MARKS)

- Write "X" where applicable.

		TRUE	FALSE
1.	In Kenya today the following are among the five most common causes of child mortality:		
	(a) Diarrhoeal deseases	[ ]	[ ]
	(b) Measles	[ ]	[ ]
	(c) Neonatal septiceamia	[ ]	[ ]
	(d) Malaria	[ ]	[ ]
	(e) Premature births	[ ]	[ ]
2.	The following are the 5 (five) general danger signs in IMCI:		
	(a) Convulsing now	[ ]	[ ]
	(b) Any history of convulsion	[ ]	[ ]
	(c) Fever	[ ]	[ ]
	(d) Not able to feed or breast food	[ ]	[ ]
	(e) Diarrhoea	[ ]	[ ]
3.	In IMCI the following are the 4 (four) main symptoms assessed for:		
	(a) Ear problem	[ ]	[ ]
	(b) Eye problem	[ ]	[ ]
	(c) Vomiting everything	[ ]	[ ]
	(d) Cough or difficulty in breathing	[ ]	[ ]
	(e) Convulsing now	[ ]	[ ]
4.	In diarrhea disease:		
	(a) Severe dehydration skin pinch (skin recoil) is slow and greater/more than two (2) seconds	[ ]	[ ]
	(b) Severe dehydration the child drinks eagerly and thirstily	[ ]	[ ]
	(c) The eyes are sunken only in severe dehydration	[ ]	[ ]
	(d) Sunken eyes disappear immediately after the child has been well hydrated from severe dehydration	[ ]	[ ]
	(e) Restlessness and irritability are signs of severe dehydration	[ ]	[ ]
5.	The following statements are true:		
	(a) The most common age for diarrhoeal disease are ➢ 6 months and < 60 months	[ ]	[ ]
	(b) "Diarrhoea" is a diagnosis	[ ]	[ ]
	(c) Dysentery is always classified as persistent diarrhea	[ ]	[ ]
	(d) "Severe persistent diarrhea" is diarrhea of duration of more than two weeks which presents with dehydration	[ ]	[ ]
	(e) The most common cause of diarrhea after age of five years are Rotavirus and Adenovirus	[ ]	[ ]

		TRUE	FALSE
6.	Ear problem:		
	(a) May be classified as mastoiditis if there is painful swelling behind the ear	[ ]	[ ]
	(b) With ear pain, pus draining from the ear or ear discharge of more than 14 days as acute ear infection	[ ]	[ ]
	(c) Is chronic ear infection when there is no pain, no pus or any discharge from the ear	[ ]	[ ]
	(d) With no pain, no pus or discharge from has no ear infection	[ ]	[ ]
	(e) Are only found in children under 5 years of age	[ ]	[ ]
7.	In nutrition:		
	(a) Nutrients are composed of macronutrients and micronutrients	[ ]	[ ]
	(b) Malnutrition is defined as inadequate nutrients than the body requires	[ ]	[ ]
	(c) Of all malnutrition, over-nutrition is the most common especially in the third world continues	[ ]	[ ]
	(d) The anthropometric measurements are weight for age, height for age, weight for height	[ ]	[ ]
	(e) The left upper circumference and head circumference are not useful assessment tools	[ ]	[ ]
8.	The following are examples of inappropriate infant feeding methods (WHO/UNICEF & NASCOP):		
	(a) Mixed feeding	[ ]	[ ]
	(b) Exclusive breast feeding	[ ]	[ ]
	(c) Complementary breast feeding	[ ]	[ ]
	(d) Complementary formula feeding and cow or goat milk	[ ]	[ ]
	(e) Exclusive replacement/formula feeding	[ ]	[ ]
9.	Down syndrome:		
	(a) Is the most common of all chromosomal disorders	[ ]	[ ]
	(b) Is a result of non-dysfunction of chromosomes which results into aneuploidy with reduced number of chromosomes	[ ]	[ ]
	(c) The fascies include flattened occiput and flattened nasal bridge	[ ]	[ ]
	(d) Small head and low set ears is not a feature	[ ]	[ ]
	(e) May result into feeding problems and reduced activity	[ ]	[ ]

		TRUE	FALSE
10.	Klinefelter syndrome:		
	(a) Is an aneuploidy of the sex chromosome	[ ]	[ ]
	(b) Is one of the chromosomal disorders called trisomies	[ ]	[ ]
	(c) Mostly present with high Intelligence Quotient (IQ) and well developed penis and testacles	[ ]	[ ]
	(d) Are usually boys and mostly eventually infertile	[ ]	[ ]
	(e) Usually have bushy bears and are short and stumpy	[ ]	[ ]
11.	Intussusception		
	(a) Is mot common in age 1 – 2 years	[ ]	[ ]
	(b) Is not the most common cause of intestinal obstruction at age of 1 – 2 years	[ ]	[ ]
	(c) Most causes are unknown but are associated with enlargement of lymphoid tissue (e.g. lymph nodes) induced by viral infections e.g. Rota and adenoviruses	[ ]	[ ]
	(d) Known causes are usually due to "lead points" e.g. Polyps & Meckel's diverticulum	[ ]	[ ]
	(e) Cystic fibrosis is not a known cause.	[ ]	[ ]
12.	The following statements are true:		
	(a) The most common type of intussusceptions is the ileocolic type.	[ ]	[ ]
	(b) The types of intussusceptions are pathologic (due to "lead points") except the ileocolic of which cause is not known but is associated with diarrhoeal disease	[ ]	[ ]
	(c) The most comfortable position in intussusception is when the child is stretched out	[ ]	[ ]
	(d) Bilous vomiting is not a feature of intussusception	[ ]	[ ]
	(e) "Currant jelly" stool and "sausage shaped" mass in the right lower quadrant are features	[ ]	[ ]
13.	The following statements are true:		
	(a) Drowning is water suffocation resulting into near death	[ ]	[ ]
	(b) Near drowning may result into death, asphyxia or laryngospasm	[ ]	[ ]
	(c) Near drowning may result into brain damage due to hypoxia	[ ]	[ ]
	(d) Multiple organ failure is a very rare a cause of death in near drowning	[ ]	[ ]
	(e) Supportive treatment in near drowning is for correction of hypoxia, hypothermia and prevention of cardiac and respiratory arrest	[ ]	[ ]

		TRUE	FALSE
14.	Bronchodilator therapy:		
	(a) May be necessary is near drowning due to laryngospasm	[ ]	[ ]
	(b) Is very useful in drowning	[ ]	[ ]
	(c) May be classified as one such is steroid therapy	[ ]	[ ]
	(d) When classified as rapidly acting bronchodilators, nebulization is referred to and not THE use of MDI (Metered Dose Inhalers)	[ ]	[ ]
	(e) I.V. aminophyllinine is only used in severe bronchospasm as in severe asthma	[ ]	[ ]
15.	Infantile hypertrophic plyloric stenosis:		
	(a) Is a genetic condition	[ ]	[ ]
	(b) Is an illness with gastric outlet obstruction	[ ]	[ ]
	(c) Is common in adults	[ ]	[ ]
	(d) Gastric outlet obstruction in children has plyloric stenosis as the most common cause	[ ]	[ ]
	(e) In barium meal shows the pathognomonic "string" or "railroad track" sign	[ ]	[ ]
16.	Hirschsprung disease:		
	(a) Is also called congenital megacolon	[ ]	[ ]
	(b) Is due to lack of ganglion cells and nerves in a segment of the colon	[ ]	[ ]
	(c) The aganglionic parts are usually in the Auerbach's plexus and not the Meissner's plexus of nerves	[ ]	[ ]
	(d) Peristalsis is not interfered with at all	[ ]	[ ]
	(e) The main symptoms are constipation or intestinal obstruction and inflamed colon (enterocolitis) which is also called "Toxic megacolon"	[ ]	[ ]
17.	Shock:		
	(a) Is an emergency only when blood pressure in very low	[ ]	[ ]
	(b) The most common shock is due to hypovolaemia	[ ]	[ ]
	(c) Cold clammy skin and thready pulse is the main symptom of endotoxic (septic) shock.	[ ]	[ ]
	(d) In shock early regulatory mechanisms are sympathoadrenal stimulation (compensatory)	[ ]	[ ]
	(e) In late shock MODS (Multiple Organ Dysfunction Syndrome) is what causes death	[ ]	[ ]

		TRUE	FALSE
18.	Poliomyelitis:		
	(a) Is an acute infection caused by poliovirus	[ ]	[ ]
	(b) Results from anterior horn cell paralysis	[ ]	[ ]
	(c) Affects adults and hence the name infantile paralysis	[ ]	[ ]
	(d) There are four types or strains and hence the vaccinations OPV_0, OPV_1, OPV_2 and OPV_3	[ ]	[ ]
	(e) Occurs in stages namely flu, meningeal and spinal stages	[ ]	[ ]
19.	In Diabetes Mellitus (DM):		
	(a) Clinically a diabetic is Type 1, Type 2 or secondary	[ ]	[ ]
	(b) Impaired Glucose Tolerance may imply "Subclinical" DM, "Asymptomatic" DM, "Latent" DM or even "Chemical" DM	[ ]	[ ]
	(c) Impared Fasting Glucose is the same as Impared glucose tolerance	[ ]	[ ]
	(d) Euglyceamia is another name for diabetes mellitus	[ ]	[ ]
	(e) MODY (Maturity Onset Diabetes of the Youth) is usually diabetes mellitus Type 1	[ ]	[ ]
20.	DKA (Diabetic Keto Acidosis):		
	(a) In an acute complication of Diabetes Mellitus only found in children.	[ ]	[ ]
	(b) Mostly occur in newly diagnosed Diabetes Mellitus Type 1	[ ]	[ ]
	(c) Infection is a leading trigger in both newly diagnosed (hitherto unknown cases) and even in known patients on treatment with insulin.	[ ]	[ ]
	(d) Is only found in Diabetes Mellitus Type 1	[ ]	[ ]
	(e) May result from higher levels of counteregulatory hormones as glucagon control and even catecholamines which are all termed as "stress hormones"	[ ]	[ ]

		TRUE	FALSE
28.	In shock:		
	(a) Fluid are given at 20 mls/kg irrespective of age or nutritional status	[ ]	[ ]
	(b) In severely malnourished in shock fluid is given at 15 mls/kg	[ ]	[ ]
	(c) In dehydration the well nourished child is transfused at 20 mls/kg and the malnourished at 10 mls/kg	[ ]	[ ]
	(d) The malnourished fluids must be given with 5% dextrose	[ ]	[ ]
	(e) In the malnourished, when can feed or by NGT ORS may be administered	[ ]	[ ]
29.	In children seizures may be caused by:		
	(a) Fever	[ ]	[ ]
	(b) Infection	[ ]	[ ]
	(c) Hypoxia	[ ]	[ ]
	(d) Cardiac arrythmias	[ ]	[ ]
	(e) Congenital brain tumour	[ ]	[ ]
30.	The following are upper neurone lesions:		
	(a) CP (Cerebral Paralysis)	[ ]	[ ]
	(b) Epillepsy	[ ]	[ ]
	(c) Guilliane Barre	[ ]	[ ]
	(d) Myasthenia gravis	[ ]	[ ]
	(e) Duchenne muscular dystrophy	[ ]	[ ]

PART 1e – ESSAY QUESTIONS

Q1. Maryanne Akinyi, 9 months old baby staying at Kisumu Ndogo with her TBA (Traditional Birth Attendant) mother developed diarrhoea four days ago and has had more than 3 bouts/day. She weighs 10 kg, has a fever and is lethargic with AVPU of <A. Capillary refill is > 2 seconds, skin pinch > 2 seconds. She has cold extremities and the radial pulse can hardly be felt.

(a)
- (i) How will you triage her?
- (ii) What is the diagnosis?

(b) She is well nourished
- (i) List down the fluids you will choose from(Give three).
- (ii) What volume of fluid will you give and how will it be given before the first assessment and when is the assessment

(c) She is anaemic with Hb < 5 g/dl.
- (i) When will you give the transfusion of blood?
- (ii) How much volume and how long?

(d) After the fourth fluid infusion radial pulse is now strong and skin pinch < 2 seconds but she can't feed.
- (i) What is your next immediate schedule for fluid therapy? (Volume, how long and next assessment)

- E N D -

KENYA MEDICAL TRAINING COLLEGE

FACULTY OF CLINICAL SCIENCES

DEPARTMENT OF CLINICAL MEDICINE

2015/2016 ACADEMIC YEAR

Final Qualifying Examinations for the Award of Diploma in Clinical Medicine and Surgery

JULY 2016

STREAM: Diploma

MODULE: PAEDIATRICS AND CHILD HEALTH PAPER I

TIME ALLOWED: 3 Hours

SESSION: Morning (09.00 – 12.00 hours)

DATE: 5th JULY, 2016

GENERAL INSTRUCTIONS

1) Write you examination number in the spaces provided on the answer booklet
2) Read instructions on answer booklet KMTC/QP-08/EAB and question paper carefully
3) Paper I accounts for 40% of the examination scores

SECTION A – SHORT ESSAY/STRUCTURED QUESTIONS – 60 marks

- This section contains **SIX questions**
- **ATTEMPT** all questions

SECTION B – LONG ESSAY QUESTIONS – 40 MARKS

- This section contains **THREE** questions
- Attempt any **TWO** questions

5 **Neonatal asphyxia can complicate to:**

a) Respiratory distress
b) Convulsions
c) Cerebral palsy
d) Hyperglycaemia
e) Diabetes mellitus

6 **In the management of spina bifida cystica**

a) A surgeon will be involved in the management
b) It takes a multidisciplinary approach
c) An abdominal ultra sound is not necessary
d) A paediatrician will be involved in the management
e) A physiotherapist will be involved in the care

7 **Concerning epistaxis in children**

a) Bleeding is always due to trauma
b) Any child with uncontrollable bleeding is admitted
c) A child may require fluid replacement or blood transfusion
d) All the children with epistaxis require admission
e) It is always due to a foreign body in the nostril

8 **Complications of typhoid fever in children include**

a) Convulsions
b) Hyperglycaemia
c) Perforation
d) Mental retardation
e) Chronic carrier state

9 Which of the following statements is True or False concerning Tb in children?

a) Full course of treatment is given to all confirmed cases only
b) Strongly suspected cases of TB are given a full course of treatment
c) The intensive phase of treatment is four months
d) HIV status should be confirmed in all children with Tb
e) Streptomycin should be reserve for multi- drug resistant TB

10 **Concerning congenital syphilis**

a) Infant may appear normal for the first few weeks of life
b) It is not preventable
c) Child will present with syphilitic rhinitis
d) Child will have lyphandenopathy
e) Laboratory investigations will yield no results

11 **Oxygen administration is important in paediatric care. The following gives an approximate 0_2 inspired by a child depending with the means of delivery used**

a) Nasal prongs.....approximately 30 – 35%
b) Naso pharyngeal catheter.....approximately 90%
c) Plain good fitting oxygen face mask .....approximately 40 – 60%
d) Oxygen face mask with reservoir.....approximately 50%
e) Short nasal catheter .....approximately 20%

12 **When admitting a child with severe acute malnutrition, the key clinical features for admission are:**

a) Oedema of both feet
b) Weight and height
c) Visible severe wasting
d) MUAC
e) Flaky paint dermatoses

22 Which of the following features below is/are suggestive of neonatal sepsis?

a) No spontaneous movement
b) Good ability to feed
c) Convulsions
d) Apnoea
e) Abnormal movements

23 Signs of severe illness in a neonate are:

a) Localised severe infection
b) Prolonged premature rapture of membranes
c) Weight loss
d) Jaundice
e) Capillary refill >3 seconds

24 A newborn weighing ≥1.5kg the feeding/fluid requirement is as follows:

a) In a well baby, feed immediately with 7.5 mls and increase by the same amount till a full day volume is reached
b) Start a sick baby on 10% dextrose on day 1 for 24 hrs
c) If the baby is just unwell, start NGT feeding on day 1
d) If the baby is on IV fluids, in day 2, use 2 parts of 10% dextrose to 1 part of half strength darrows solution
e) Expressed breast milk is not mandatory

25 During neonatal resuscitation

a) Meconium aspiration has nothing to do with the baby not breathing at all
b) If the baby has meconium aspiration dry and stimulate fast then do suction
c) Breathing of a newborn should be started within 60 seconds
d) If anything is visible in the airway use suction to clear
e) The baby's head should be put in neutral position

26. The following are anxiety disorders in children and adolescents

a) Separation anxiety
b) Phobia
c) Post traumatic stress disorder
d) Mania
e) Schizophrenia

27. The management of conversion syndromes includes

a) Reprimanding the child to stop pretending
b) Threatening the patient to see if there is a change in behaviour
c) Take a good psychiatric history to reveal the source of conflict
d) Thorough physical examination to exclude an organic problem
e) Counselling and behaviour modification

28. The management of conduct disorders involve

a) Behaviour modification
b) Mentorship recreational programmes
c) Legal sanctions
d) Involvement of family
e) Involvement of relevant authorities

29. Clinical features of sickle cell disease include

a) The disease starts at the age of 6 years
b) Foot and mouth syndrome
c) Impaired growth and development
d) Acute chest syndrome
e) Stroke

30. Further measures on the management of sickle cell disease crises include

a) Give intravenous fluids
b) Infections should be left alone for they have no harm
c) Children with moderate to severe pain should be given dihydrocodeine or codeine phosphate
d) Ibuprofen is preserved for severe pain
e) Hydroxyl urea is a very strong analgesia for sickle cell disease